



جامعة الإمام عبد الرحمن بن فيصل
IMAM ABDULRAHMAN BIN FAISAL UNIVERSITY



1st Quarter Report CY 2026

NURSING QUALITY IMPROVEMENT UNIT

Vivien B. Go

QUALITY IS DOING THE FIRST THINGS RIGHT FIRST TIME AND EVERY TIME
PHILIP CROSBY

The Key Performance Indicators FY 2025

1. Falls Rate per 1000 Patient Days (Outcome)
2. Hospital Acquired Pressure Injury Rate (Outcome)
3. % Nursing Compliance to Patient Identification during Medication Preparation (Process)
4. % Nursing Compliance to Patient Identification during Medication Administration (Process)
5. % Nursing Compliance to Correct Handover using SBAR Tool (Process)
6. % Compliance to Complete Fall Risk Assessment & Reassessment (Process)
7. % Pediatric IV Infiltration Rate (Outcome)
8. % Adult IV Infiltration Rate (Outcome)
9. % Adult/Pediatric Extravasation Rate following Medication Administration (Outcome)
10. Staff Ratio in Critical Care Areas (Structure)
11. % Compliance to Blood & Blood Products Transfusion Policy (Process)
12. % Compliance to Pain Screening (Process)
13. % Compliance to Pain Assessment, Intervention, & Reassessment (AIR Cycle) (Process)
14. Multidisciplinary Key Performance Indicators
 - a. Medication Related Incidents (Process/Outcome)

Falls Rate/1000 Patient Days (Outcome)

Reported fall incidents from January – February 2026, the sum total of 20 falls across nursing units.

Benchmark: Internal <2.1

Formula: N/D multiply by 1000

Numerator: Number of patient falls (with or without injury)

Denominator: Number of Patient Days

Multiply by 1000

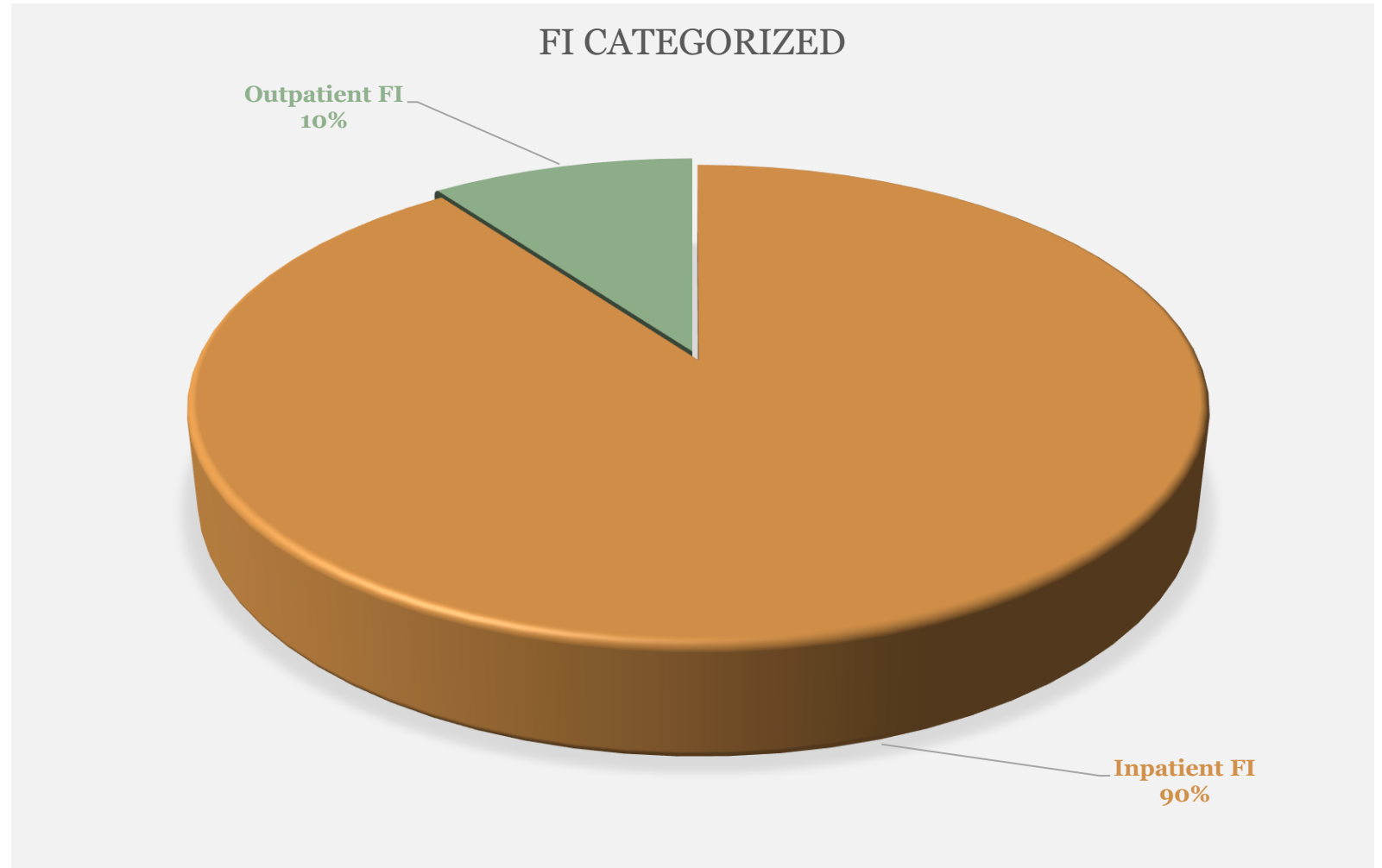


Table 1 represents the fall categorized as inpatient and outpatient fall. Inpatient and ambulatory fall incident reporting is a critical, proactive safety mechanism in healthcare designed to reduce preventable harm, improve clinical outcomes, and mitigate significant financial, legal, and operational burdens on health systems. www.nih.gov

Comparison of the previous calendar year's performance with the recent results to show if there are improvements. Although, fall incident is multifactorial, this will remind for further enhancement of any preventive measures to reduce fall incidents.

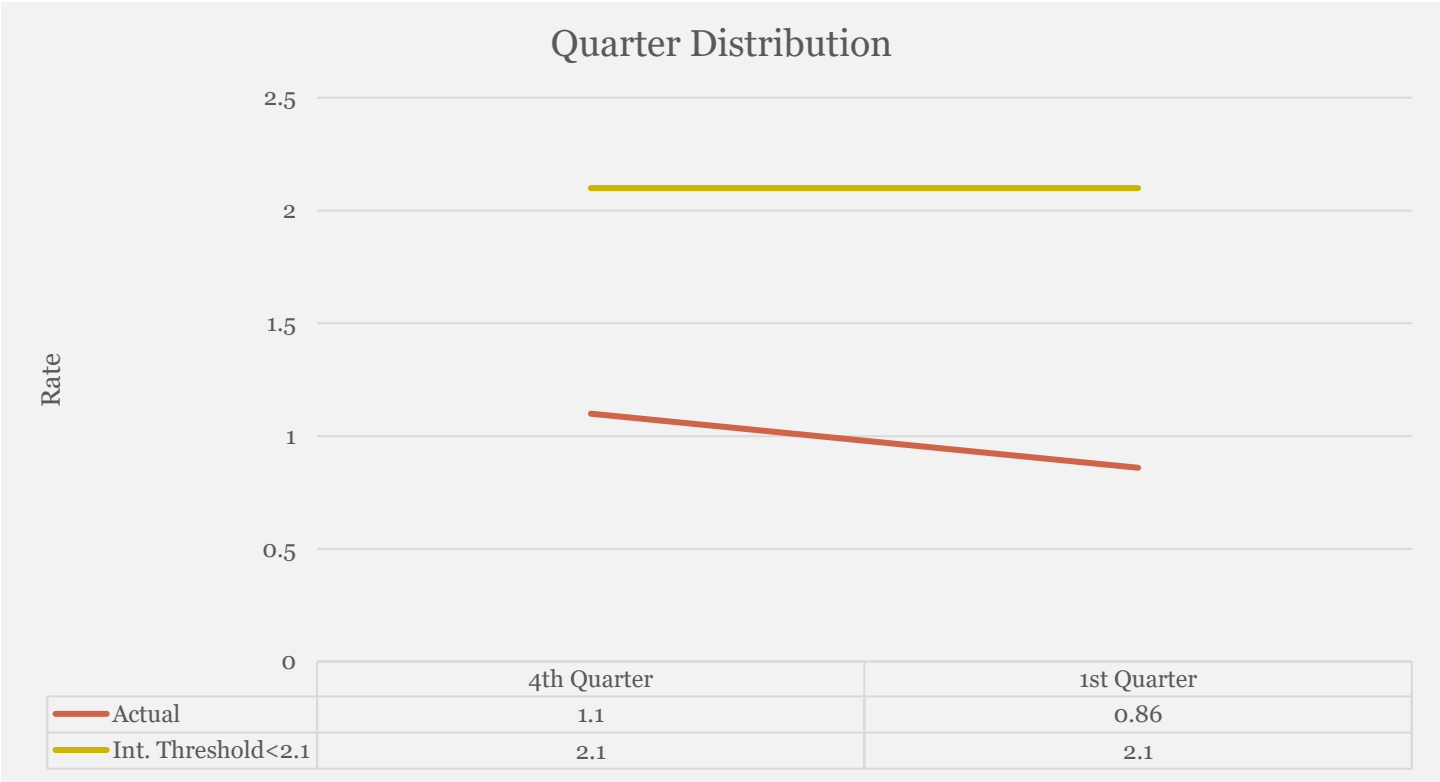


Table 2 Implies a decreased in the recent quarter. Reporting FI enables Root Cause Analysis (RCA) to understand *why* a fall happened, shifting focus from blame to enhancing care processes. National Institute of Health (NIH) (.gov) www.nih.gov

FALLS RATE/1000 Patient Days

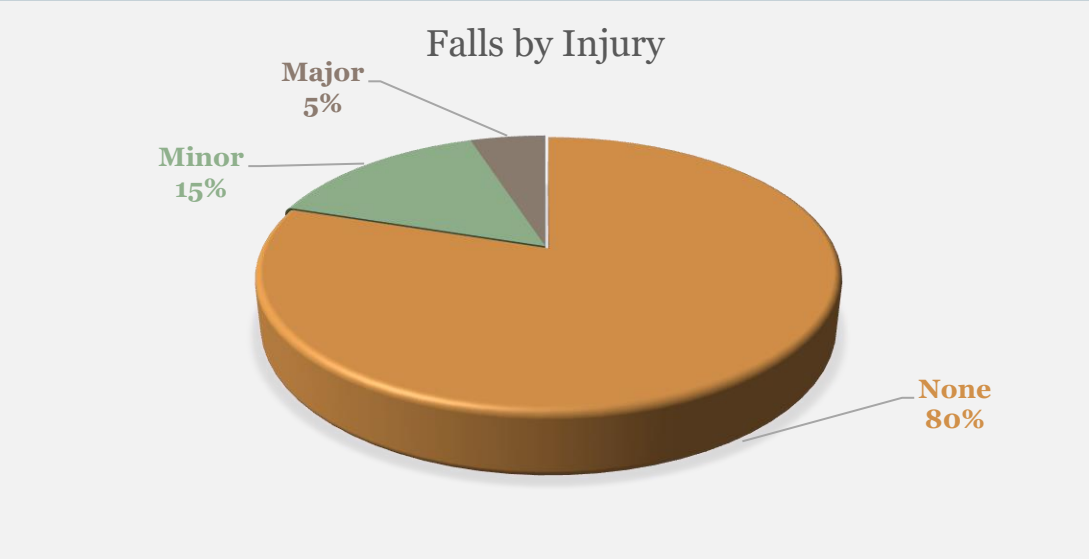


Table 3 Reporting fall incidents categorized by the severity of the injury is a critical patient safety and risk management tool, primarily used to identify patterns, prevent future occurrences, and ensure appropriate care. By analyzing the relationship between the fall and the injury—ranging from no injury to moderate/major injuries (e.g., fractures, traumatic brain injuries)—organizations can implement targeted, data-driven interventions to enhance safety. www.nih.gov

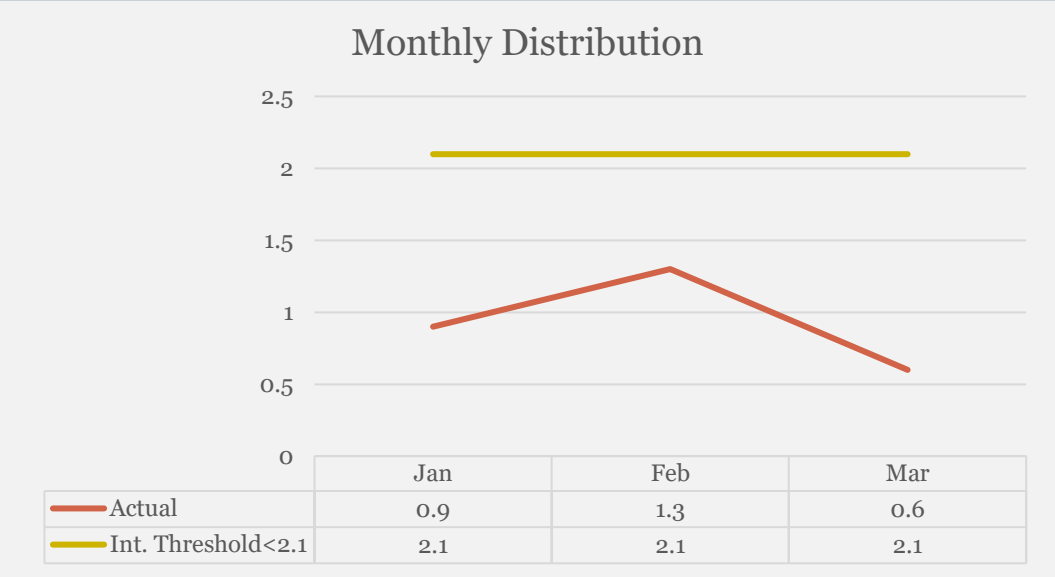


Table 4 The line chart shows the trends if there are increased or decreased fall rate from all months of the quarter. A slight increase during the month of Feb, however, still below the threshold.

Contributory Factors

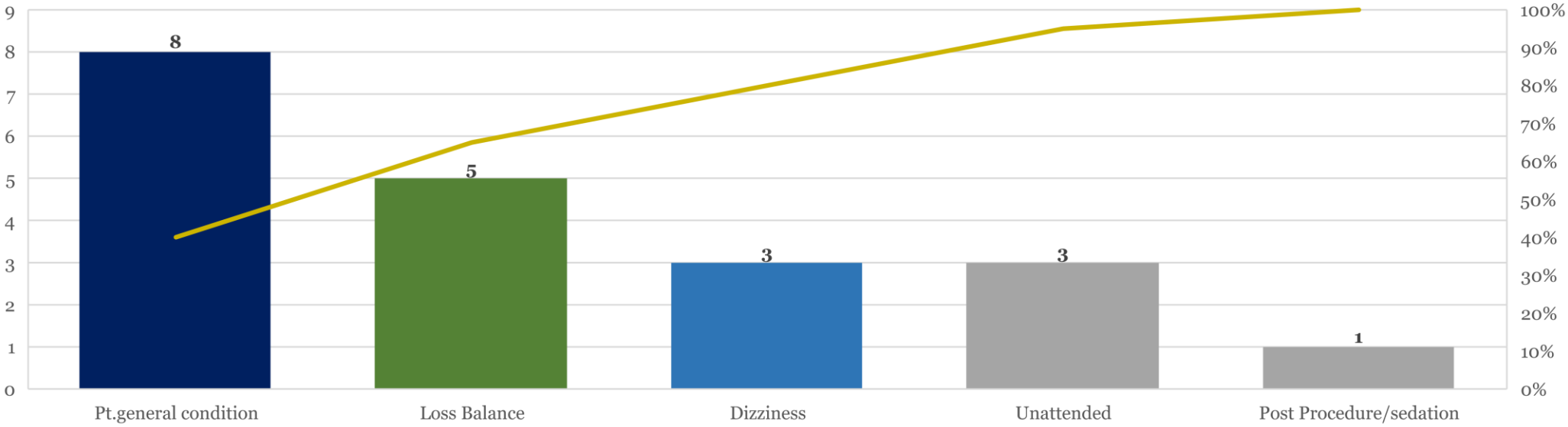
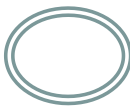


Table 5 the Pareto analysis highlights that most falls were associated with intrinsic patient-related factors, particularly patient general condition, loss balance, and dizziness. Targeted interventions focusing on early risk identification, mobility assistance, and environmental safety are essential to reducing fall incidents and strengthening patient safety outcomes. AHRQ v2024

Falls per 1000 Patient Days

Distribution of FI by Time (24-hour Cycle)

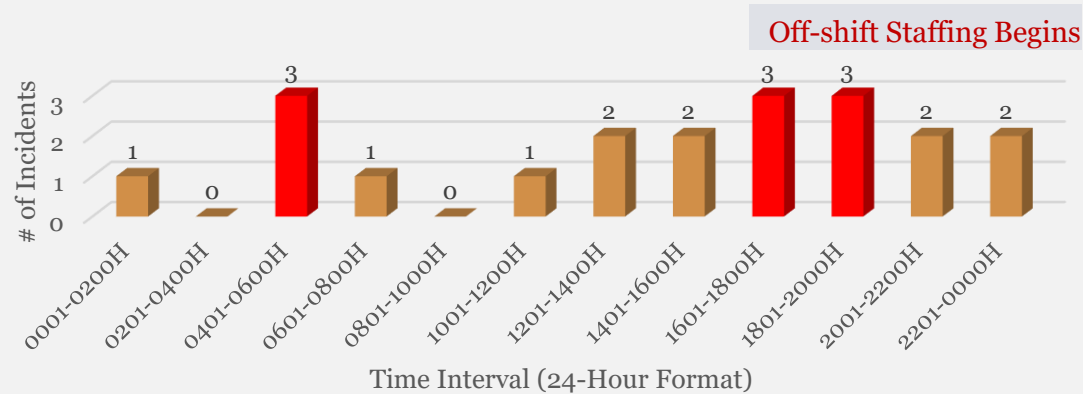


Table 6 implies that **30% of total falls**. The spike at 1801–2000H directly aligns with the transition to "off-shift" staffing. With a patient-to-staff ratio of **10:1**, the bedside team is physically unable to provide the "eyes-on" supervision required during high-activity hours (dinner, medication passes, and bedtime routines). While secondary peak (**3 incidents**) represents a critical risk window. This is typically when patients attempt unassisted "self-toileting" upon waking, occurring at a time when staff fatigue is highest and shift-change preparations often begin. *Sources: Aiken et al., The Studer Group / Mitchell et al.)*

Age Distribution

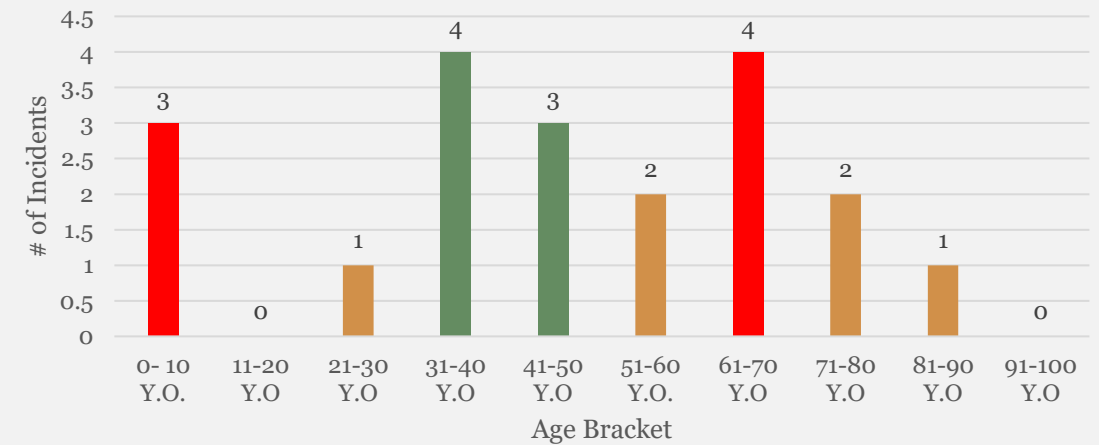


Table 7 Primary Peaks: The highest incidence occurs in two distinct groups: 31–40 Y.O. and 61–70 Y.O. (4 incidents each). **Vulnerable Pediatrics:** The 0–10 Y.O. group is the third-highest (3 incidents). This is a high-risk area because pediatric falls are often viewed as a major "failure of surveillance" by regulators. **The "Active Adult" Spike:** The spike in the 31–40 and 41–50 age groups (7 total falls) is often related to patients who feel "overconfident" in their mobility or are trying to maintain independence without calling for help—exactly when staff are busy with other patients.

Falls by Location

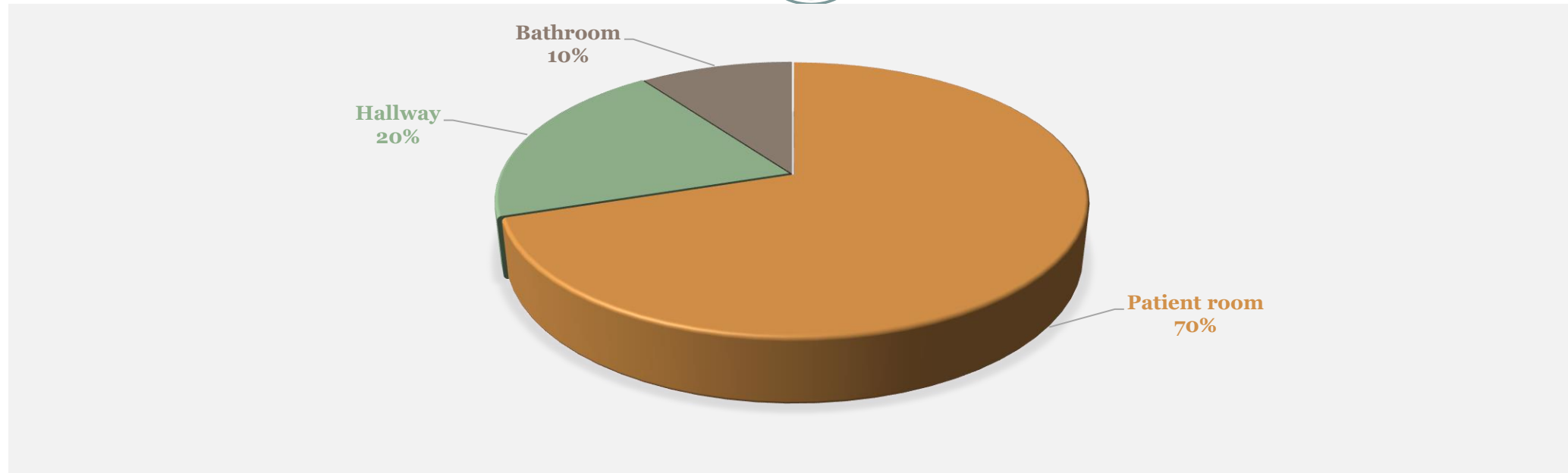


Table 8 indicating highest in patient room 70% wherein most of the activities occurs. Patients spend the vast majority of their hospital stay in their rooms, increasing the statistical "opportunity" for a fall to occur there compared to the relatively brief time spent in any other areas of the hospital. (<https://www.ncbi.nlm.nih.gov>)

Gender Distribution

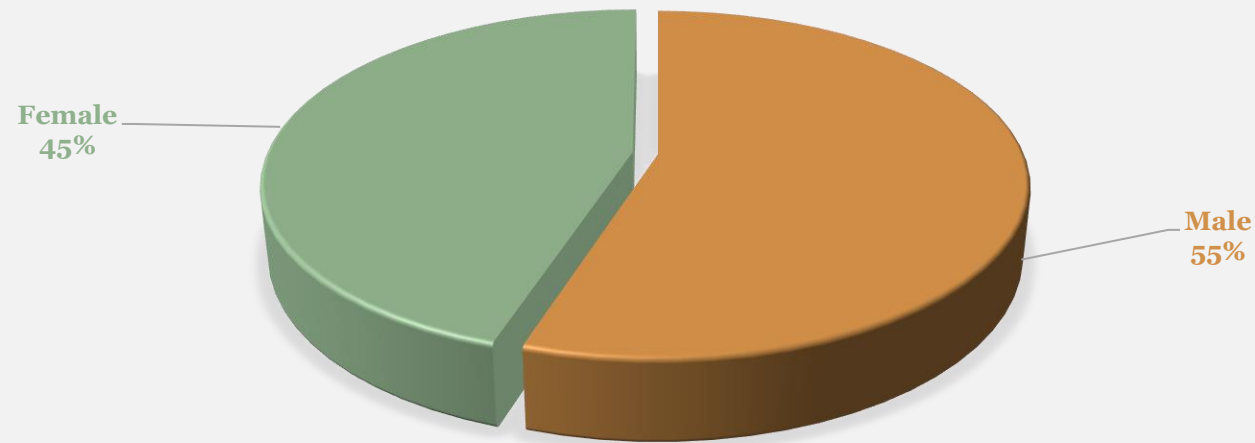
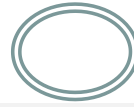


Table 9 gender (men/women) fall risk differs by studies and statistical data. Out of 78 falls, 55% identified among male, and 45% among female. Congruent to the cause of fall (loss balance 25%-top 3 highest cause), men more likely than women to fall due to loss of support with an object, moving into a seated position versus getting up or rising (mindset that they are still able/stronghold), physical strength & stability; risk-taking & independence- (PMC-PubMed Central).

Unit Distribution



FI by Unit Distribution

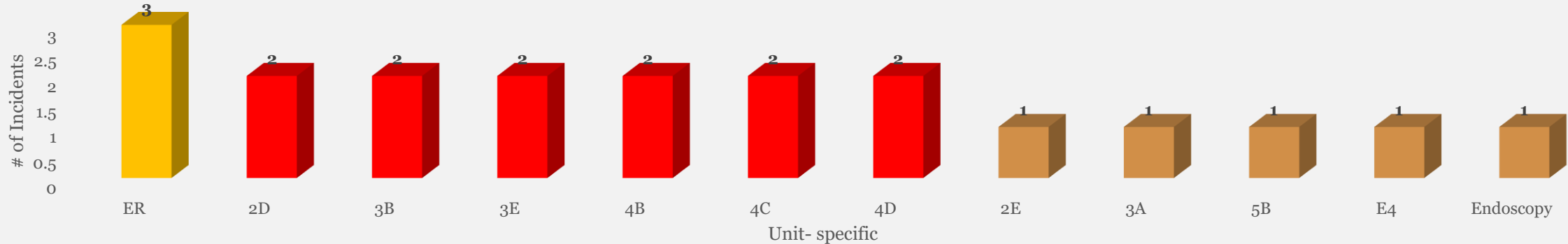


Table 10 High Patient Acuity: Units 2D, 3B, 3E, 4B, 4C, and 4D are currently managing higher-complexity patients. The data suggests that current safety protocols on these units are being strained by this increased clinical demand. **Off-Shift Staffing Vulnerability:** Internal reviews indicate that the majority of these incidents occur during "off-shifts" (nights and weekends). The current staffing ratios during these periods are insufficient to manage the high-acuity needs, creating a "safety gap" that leads to increased notifications. *Aiken, L. H., et al. (2011). "Nurse Staffing and Education and Hospital Mortality." Medical Care.*

% Completion of Fall Risk Assessment & Reassessment(Process)

Actual: 77%

Target: ≥95%

Int. threshold: 88%

Fla. N/D X 100 wherein

Numerator: # of Completed Fall Risk Assessment tool

Denominator: Total # of Fall Risk Assessment Tool Reviewed
Multiply by 100

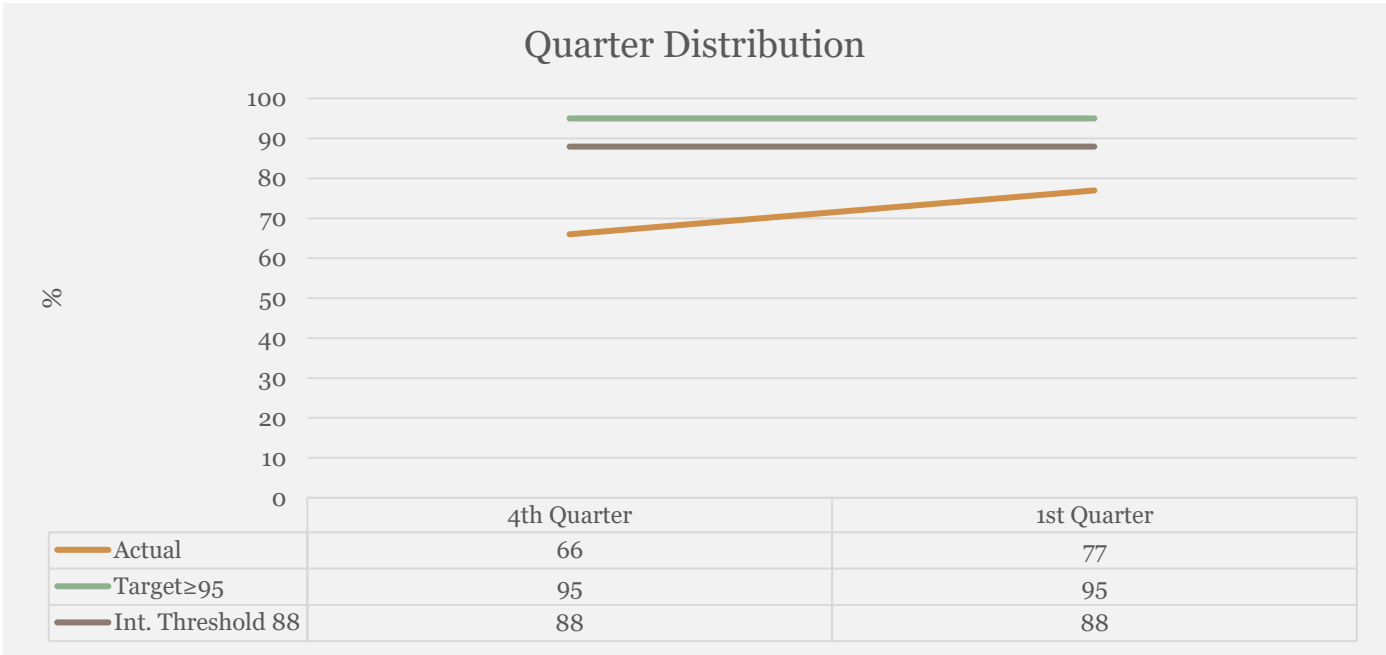


Table 11 graph shows comparison between Q 4 2025 & Q1 2026 indicating both years are below the approved and set target. Fall risk screening, assessment & reassessment is crucial for the prevention of fall incidents. This includes but not limited to reminder to patients on use of call bells whenever they need assistance on ambulation, reeducation (patients/significant others), ensuring safe environment i.e., periodic toileting rounds, (side rails up/no clutters/appropriate lightings/no wet floors/bed in low level). On the other hand, frequency of reassessment is tantamount to prevention/implementation of safety precautions.

Overall Compliance

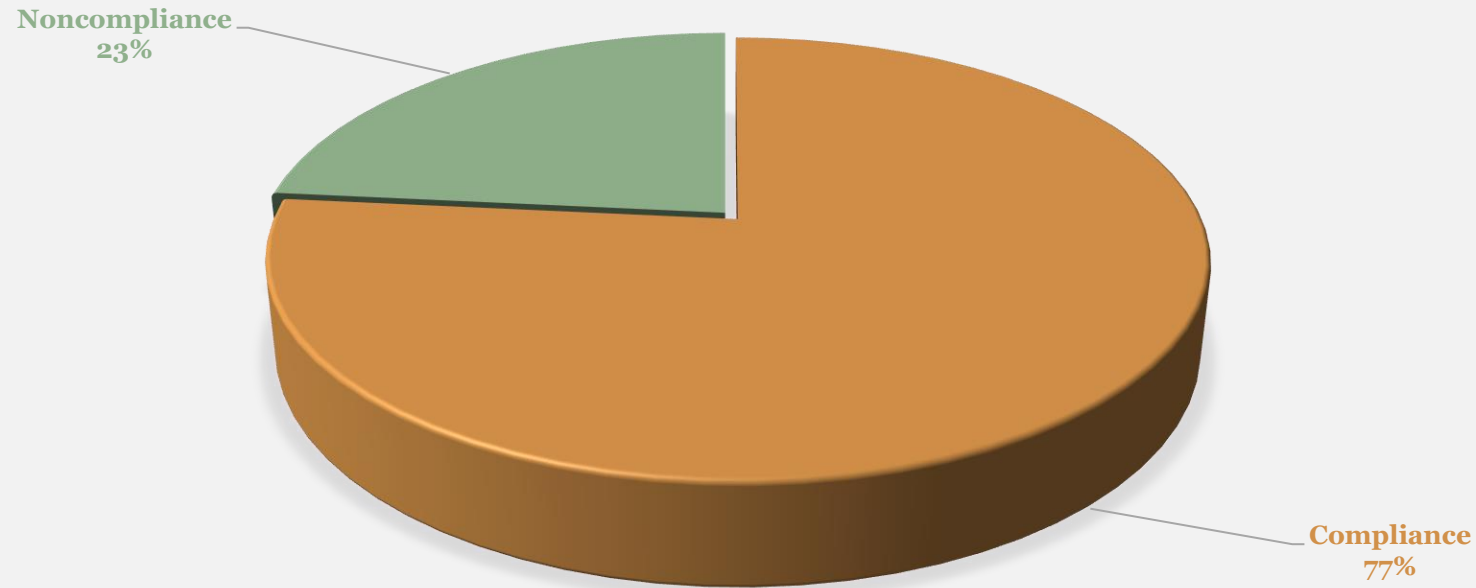


Table 12 represents the overall compliance of the department showing below the target. A challenge for both the unit managers and designees (leadership accountability) to find strategies in strengthening awareness, ensuring compliance and empowering staffs for ownership.

Falls Rate/1000 Patient Days

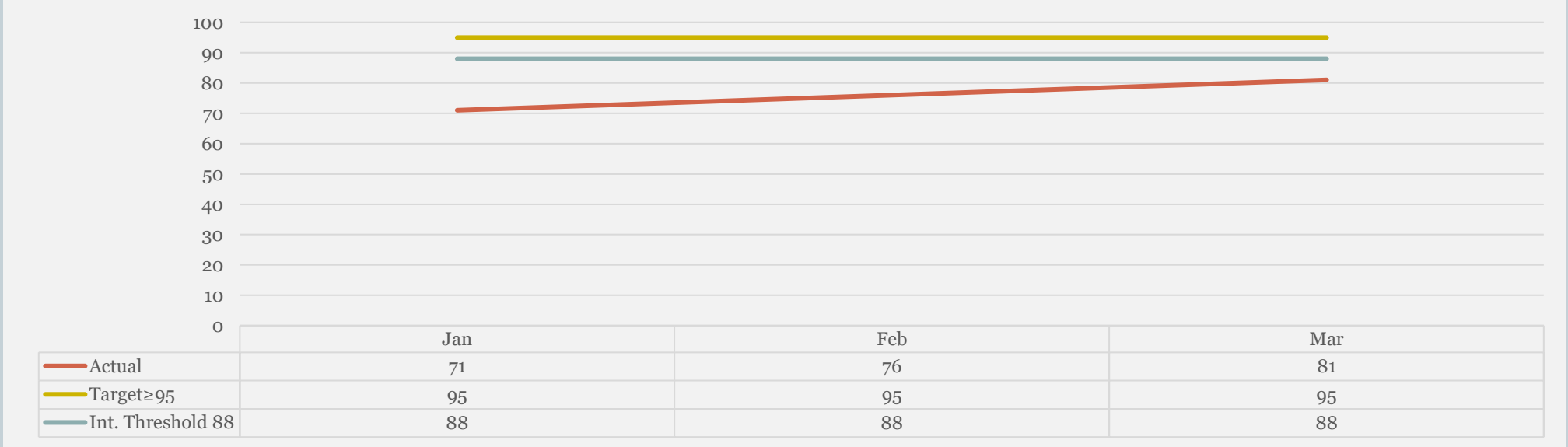


Table 13 graph represents the compliance by month to fall risk assessment & reassessment wherein all indicates a need to continuously monitor & reeducate by the unit managers and designees (leadership accountability). Engage unit managers and charge nurses, include indicator in unit performance review.

Causes of Noncompliance

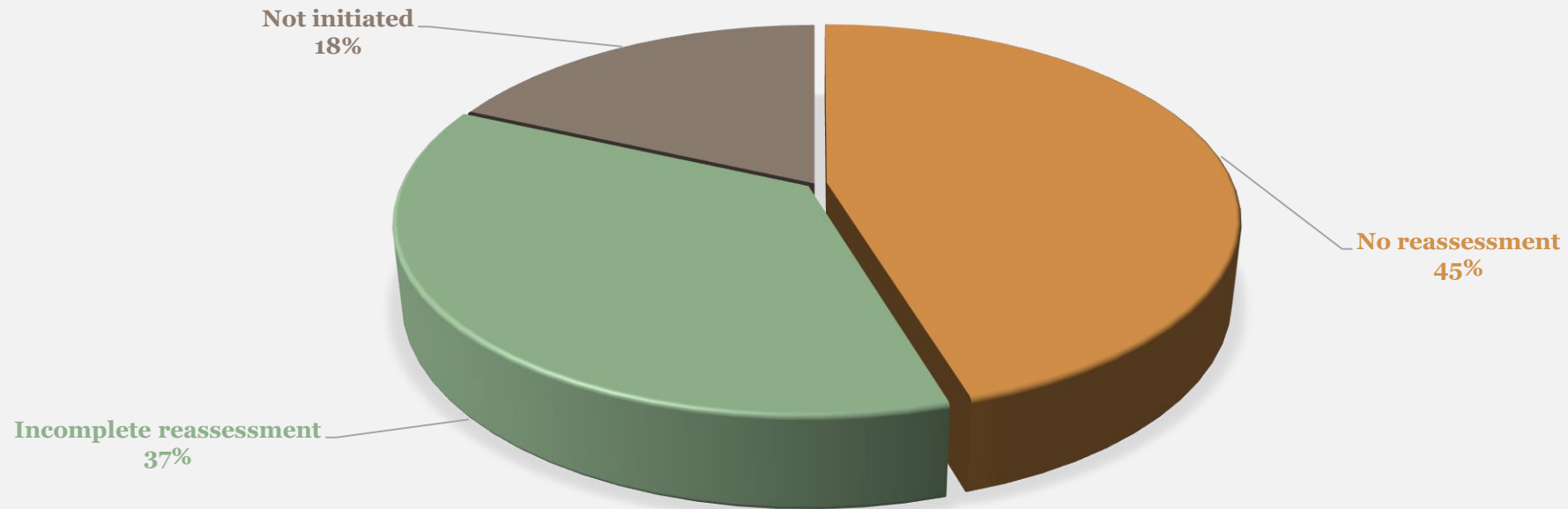


Table 14 graph represents the causes of incompliance wherein 45% shows no reassessment, 24% incomplete reassessment, & 18% not initiated respectively. All the three (3) causes impact the importance of identifying, mitigating and preventing risk.

Unit Distribution

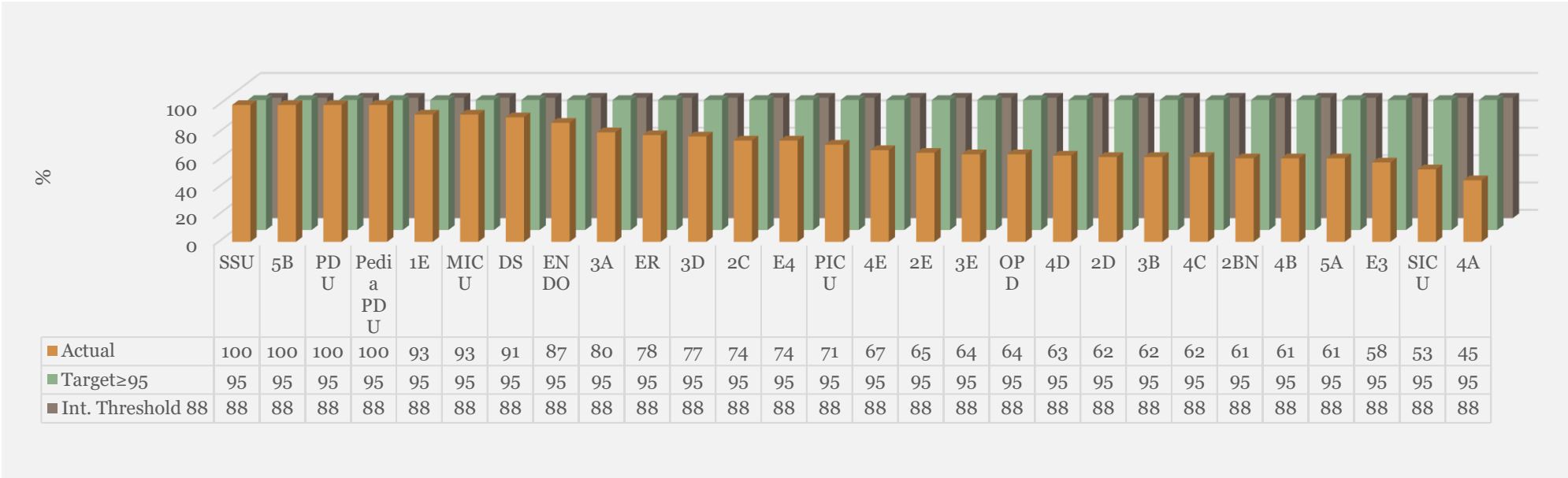


Table 15 graph represents that out of 28 nursing units, only 14% are above the target & int. threshold. Actual performance of the remaining twenty four (24) units indicate a need to focus on the possible cause of incompliance and strategy on how to improve performance.

Action Taken/Next Steps:



1. Core Philosophy & Staff Awareness

- Universal Risk Mindset: Every patient should be treated as "probably at risk." Screening, assessment, and reassessment are mandatory, not optional.
- High-Risk Focus: Increase awareness of specific triggers like age, gender, mobility (gait/balance), and the need for adult supervision for children.

2. Operational Compliance (The "Fixes")

- Mandatory Reassessment: Since a lack of reassessment is the primary cause of non-compliance, it must be integrated into every shift handover.
- Toileting Rounds: Despite staffing challenges, regular "checking and observing" for toileting remains the top priority for prevention.
- Data Validation: Unit managers must use random sampling to ensure data accuracy and keep their teams informed of their performance.

3. Clinical Protocol & Standards

- Investigation Protocols: Any fall resulting in injury (NDNQI classified) or repeat falls must trigger a formal Root Cause Analysis (RCA).
- The "5 P's" Strategy: Standardize prevention using the 5 P's: Pain, Potty, Periphery, Position, and Pump.
- Interdisciplinary Ownership: Reactivate the Fall Prevention Task Force for clinical rounds and tracer activities to ensure safety is a shared responsibility.
- System Integration: Implement automated alerts in the Health Information System (HIS) after IPSG approval-**done**.

4. Accountability & Standards

- Manager Checks: Unit managers will perform random sampling of patient files to ensure assessments are being done correctly.
- Team Responsibility: Fall prevention is a multidisciplinary task. Expect visits from the Fall Prevention Task Force for clinical rounds and "tracer" activities.
- JCI Compliance: Remember that accurate fall assessment is a core part of our JCI 8th Edition (AOP Standard) accreditation.



Hospital Acquired Pressure Injury Rate (Outcome)

Reported HAPI: 93

Actual: 4.65

Benchmark: Internal – Establishing Baseline

Fla: N/D X 100 wherein

Numerator: # of HAPI in a month

Denominator: Total patient days
Multiply by 1000

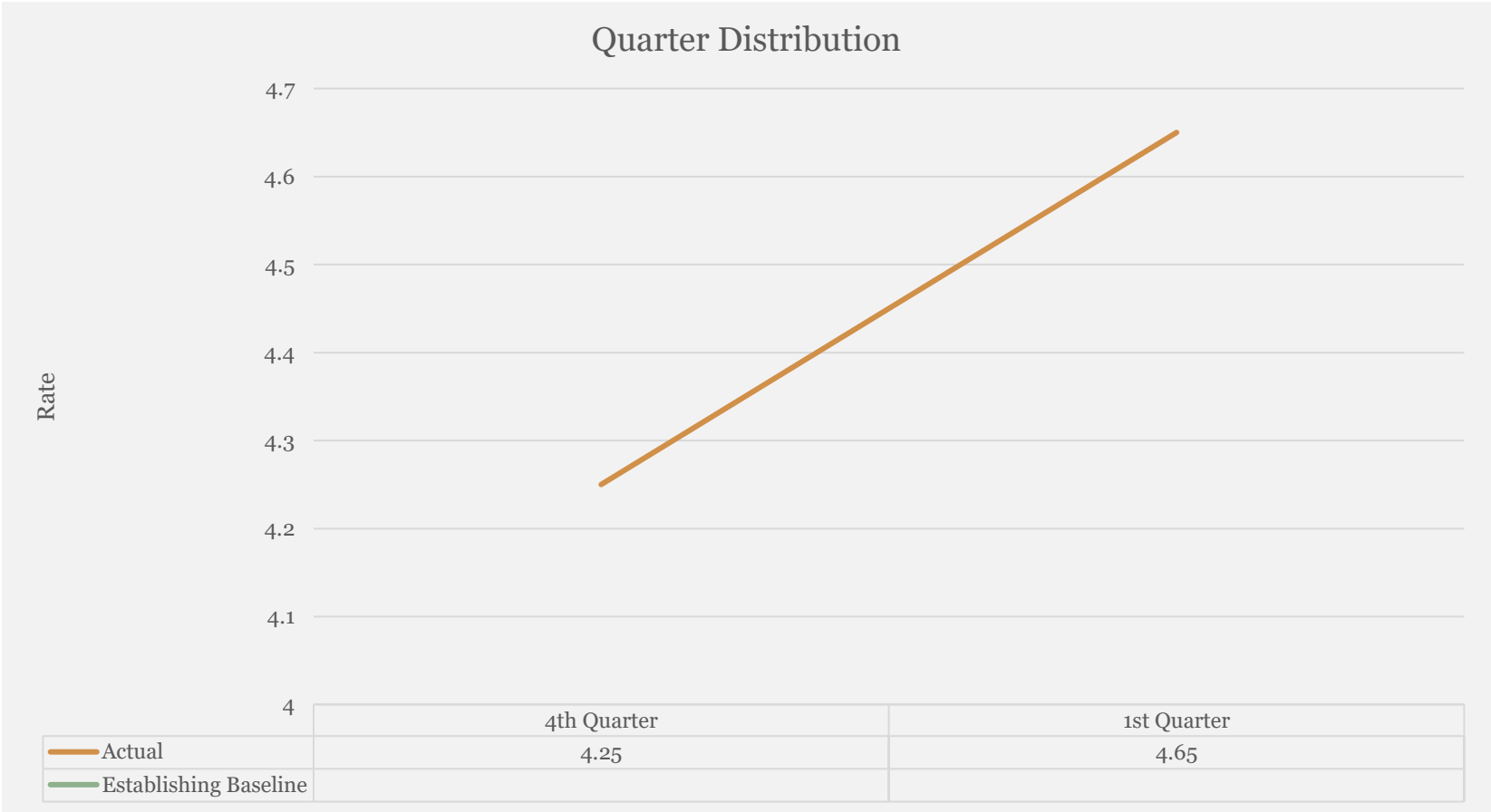


Table 16 While Q1 2026 shows a 9.4% increase in HAPI rates compared to the Q4 2025 baseline, this period marks our first year of fully stabilized 'Patient Day' reporting. The current variance of 4.65 will be audited against our internal gold standard of 1.4 to identify and mitigate the operational risks contributing to this climb.

Hospital Acquired Pressure Injury

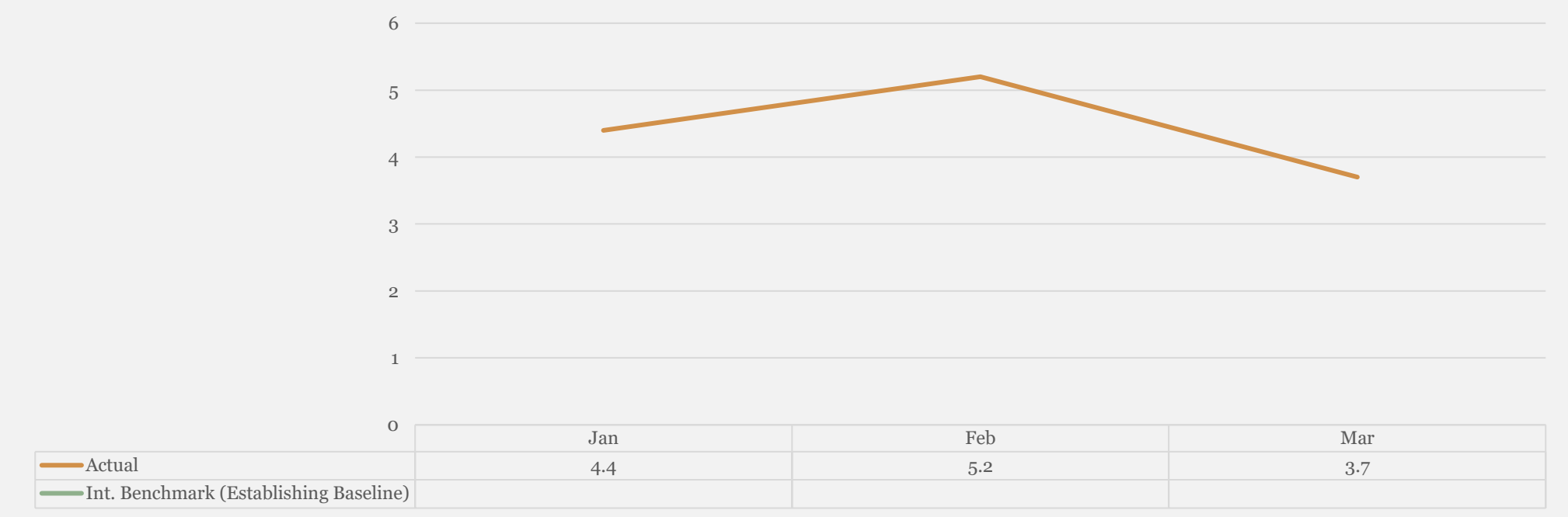


Table 17 line chart represents all months of the year showing a remarkable increased for the months of Jan, February & March. Using 2025 as internal threshold baseline with a gold standard to be identified and upcoming, continuous monitoring, timely assessment & use of preventive bundles should be strengthened in risk mitigation.

Hospital Acquired Pressure Injury Rate

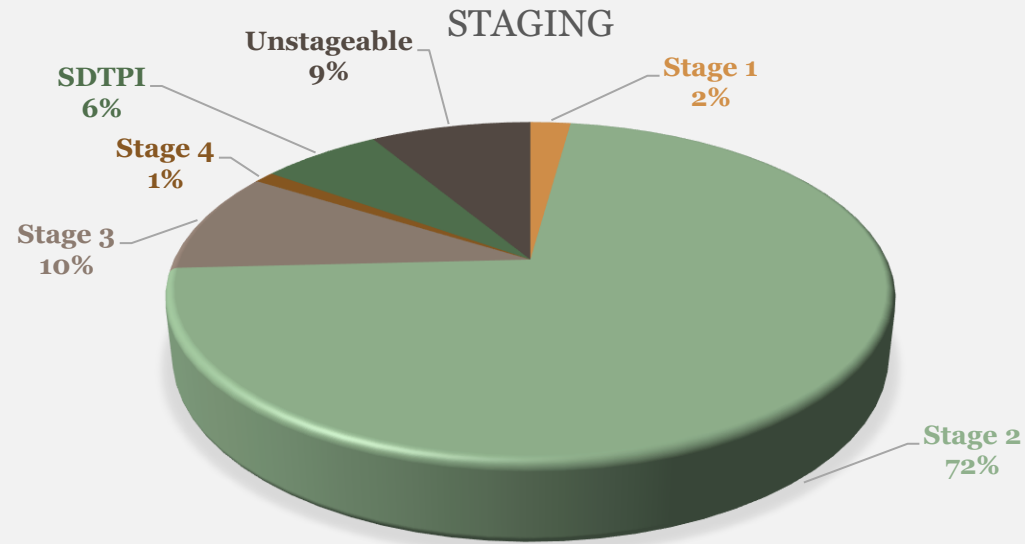


Table 18 shows percentage of pressure injury by stages wherein stage 2 garnered the highest 72%, followed by stage 3 with 10% and 9% unstageable respectively. To implement an appropriate intervention, correct staging helps in mitigating risk and managing the injury.

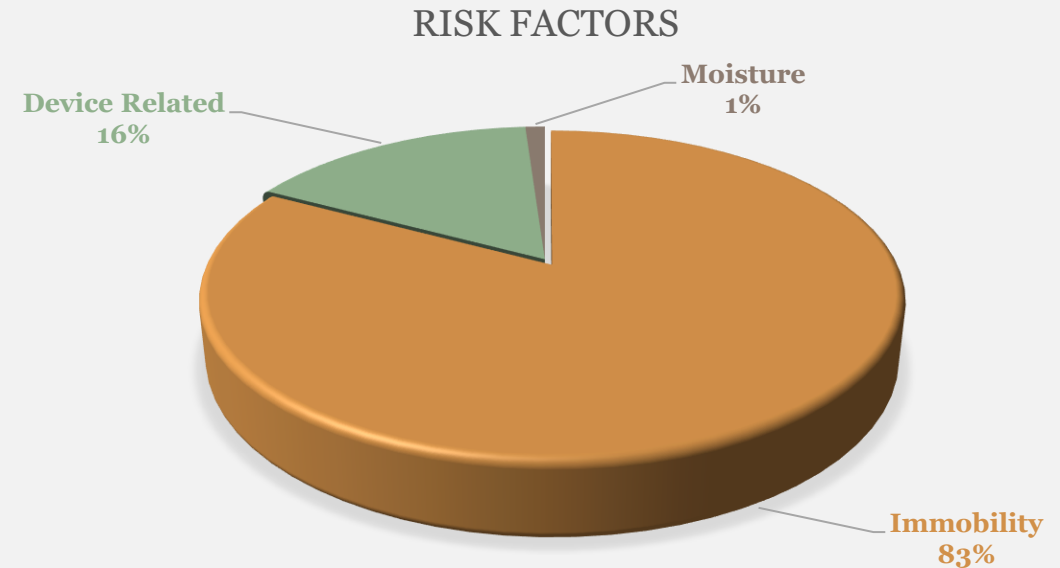


Table 19 factors both extrinsic & intrinsic contributes to the occurrence of impairment implies higher incidents among long-staying, bedfast, (congruent to the affected sites, and reported unit specific PI incidents).

Hospital Acquired Pressure by Sites

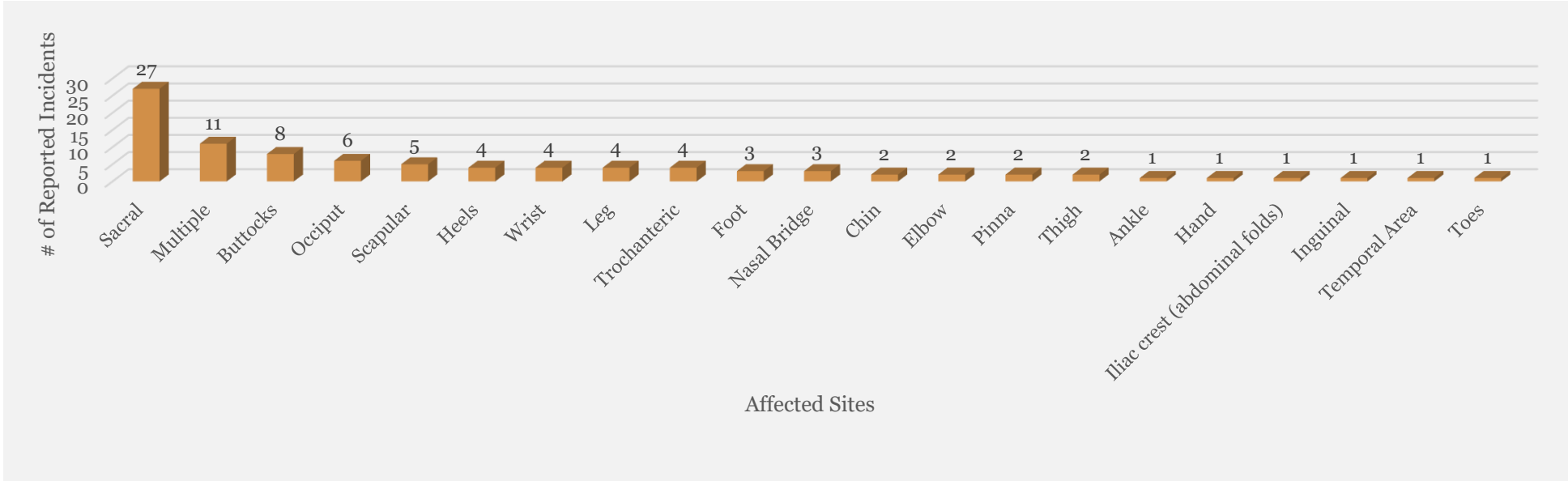


Table 20 top three (3) highest implies that most reported pressure injury happened to those patients confined on the bed or bedfast. PIs result from long periods or repeated pressure applied to the skin, soft tissue, and the external pressure exceeds capillary pressure. Therefore, bed-bound individuals are more prone to develop PI (ncbi.nlm.nih.gov).

Unit Distribution

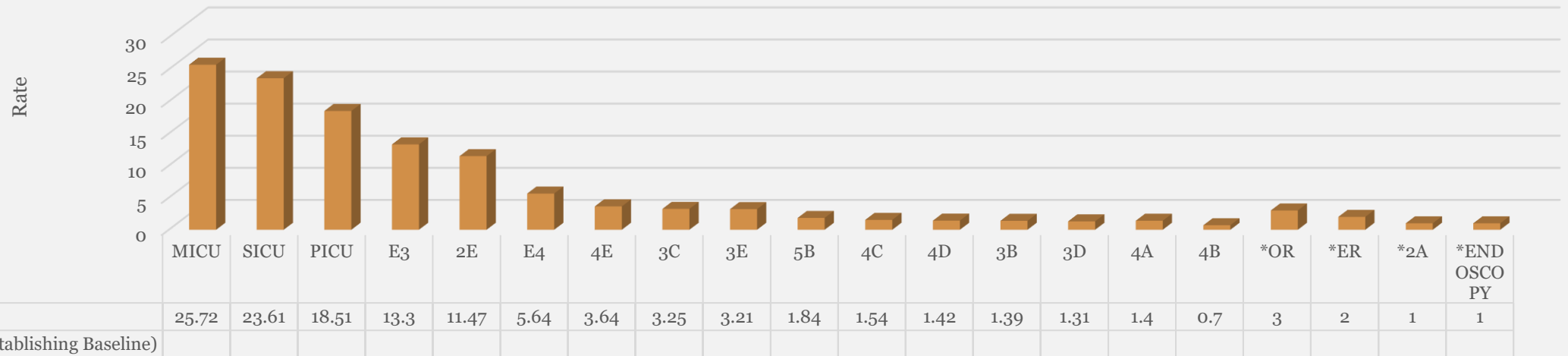


Table 21 top 3 highest among critical care units implies that most reported pressure injury happened to those nursing units with patients who are bedfast/prolonged stay on bed. Pressure injuries result from long periods of repeated pressure applied to the skin, soft tissue, and the external pressure exceeds capillary pressure. Therefore, bed-bound individuals are more prone to develop PI (ncbi.nlm.nih.gov).

Note: “Rates for Inpatient Units are per 1,000 Patient Days. **Proposed:** Rates for OR, ER, 2A, & Endoscopy are to be calculated per 1,000 Visits/Procedures to account for the nature of throughput areas.”

Action taken/Next Step:

Strategic Action Plan: HAPI 2nd Cycle PI Project (2026)

I. Clinical & Staffing Optimization

- Acuity-Based Repositioning: Implement "Unit Turning Teams" and standardized Clock Turning protocols to mitigate the impact of staffing ratios on high-dependency units.
- Targeted Re-education: Continue the "HAPI Project" collaboration between Wound Care (WC), Staff Training, and NQI to sustain the positive reduction in incident rates.

II. Multidisciplinary Clinical Support

- Standardized Referral & Assessment: Mandate HIS-based referrals for all HAPI stages to the Wound Care Nurse (WCN). This ensures expert intervention, appropriate dressing selection, and direct assessment within 24 hours.
- Multidisciplinary Periodic Rounding: Resume structured bedside rounds involving clinicians, nursing, and WCNs to ensure holistic management of complex, multifactorial cases.

III. Material & Resource Management

- Advanced Support Surfaces: Finalize the procurement and deployment of high-tech positioning devices (e.g., Tortoise System) and specialized skin barriers to address the second highest cause of injuries (Device-Related/MDRPI).
- Supply Chain Continuity: Collaborate with Materials Management to ensure the consistent availability of prescribed dressings and preventative skin-care products.

IV. Engagement & Digital Transformation

- Patient/Family-Centered Care: Strengthen multidisciplinary education to gain patient and family cooperation in repositioning schedules and skin-safety protocols.
- HIS Integration: Transition to the updated HAPI Screening/Assessment tool in the new HIS. This will enhance the ability of Nurse Managers to track wound status and document multidisciplinary care in real-time.

V. The 2nd Cycle HAPI PI Project 2026- revision of P/P, clarified processes (inpatient regular & critical algorithm specific), and the implementation of preventive bundles.

VI. Collaborative Initiative by the Department & the Monlycke – HAPI Project towards “Excellence” addressing the program from preventive bundles implementation, securing supplies, training, and continuous monitoring.



**% Nursing Staff Compliance
to Patient Identification
during Medication
Preparation(Process)**

Actual: 93.5%

Target: 100%

Internal Threshold: 98%

N/D multiply by 100 wherein

Numerator- Number of nurses
observed compliant with patient
identification policy during
medication preparation

Denominator - Total number of
nurses observed

Multiply by 100

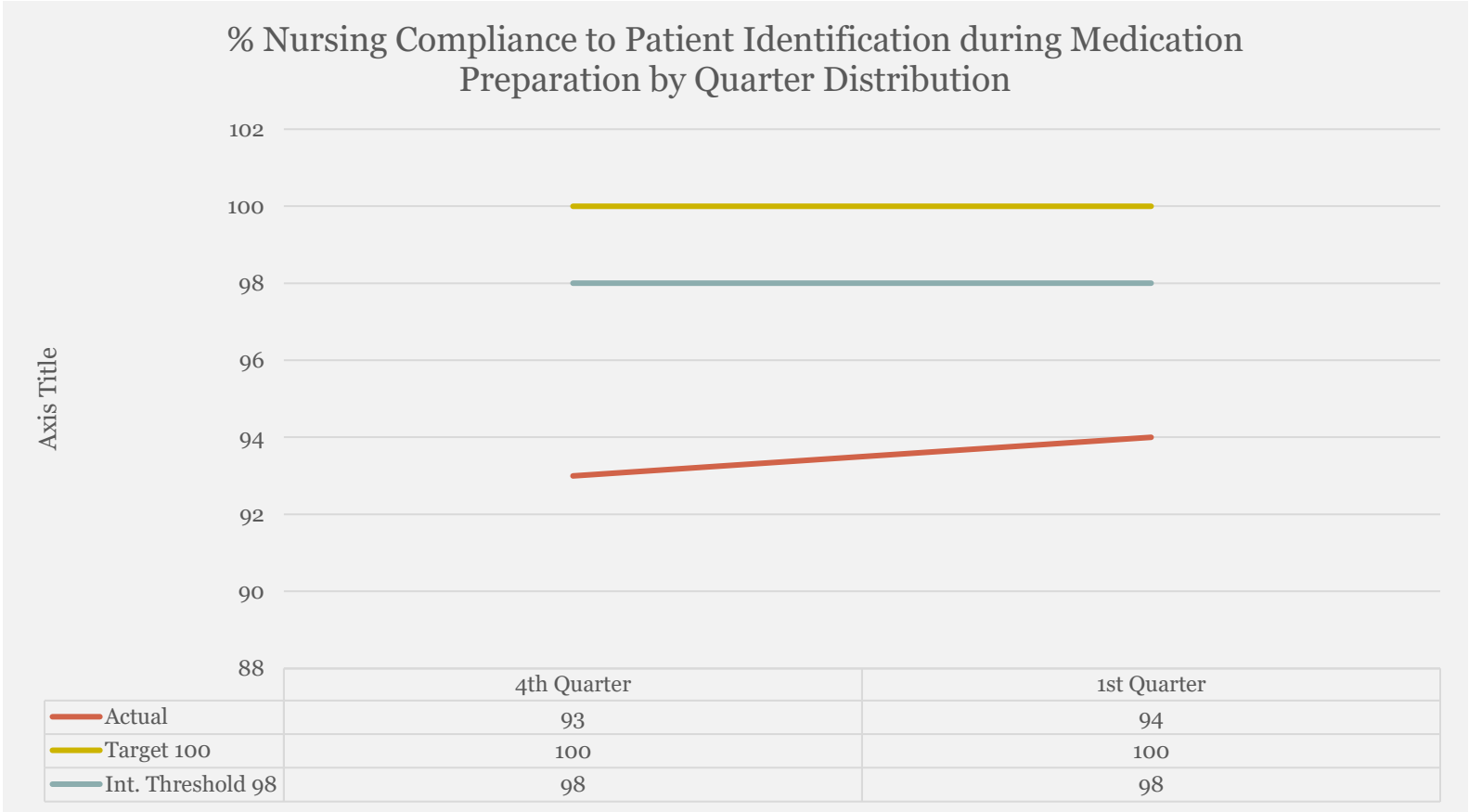


Table 22 a comparison between the 4th quarter 2025 and 1st quarter 2026. It implies that both quarters are below the target and internal threshold. Correct patient identification is mandated in all patient care services rendered (*8th Edition JCI Standards for Hospitals, 1 Jan 2025*).

Overall Compliance

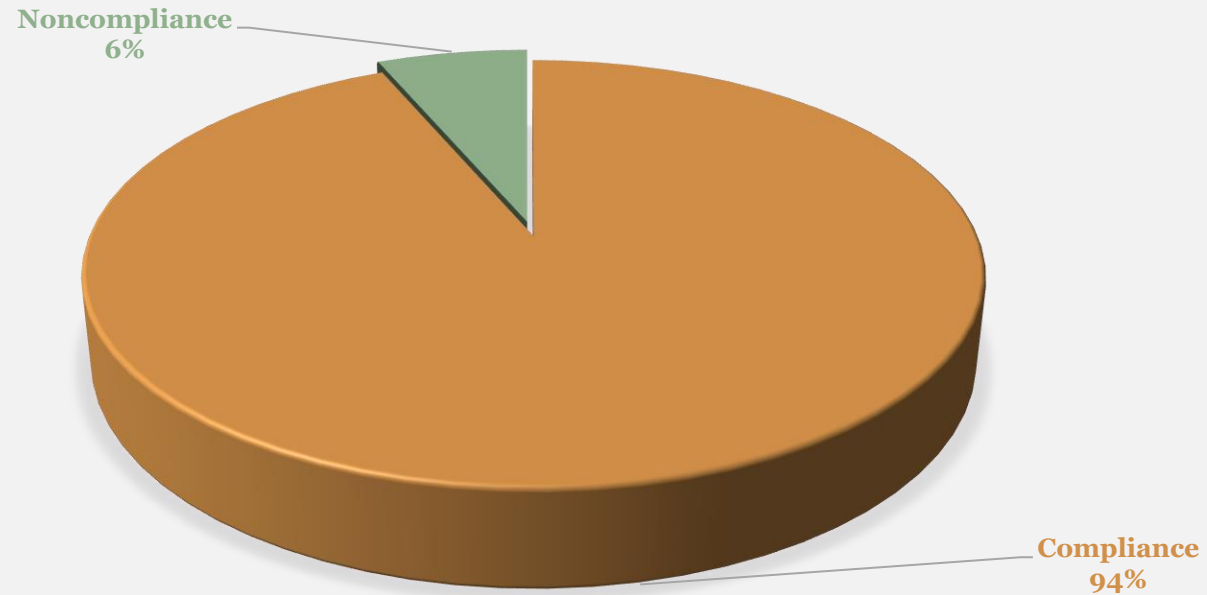


Table 23 indicates low performance against the set target of 100%. Strengthen the awareness on how to improve and what strategies can be done.

MONTHLY DISTRIBUTION

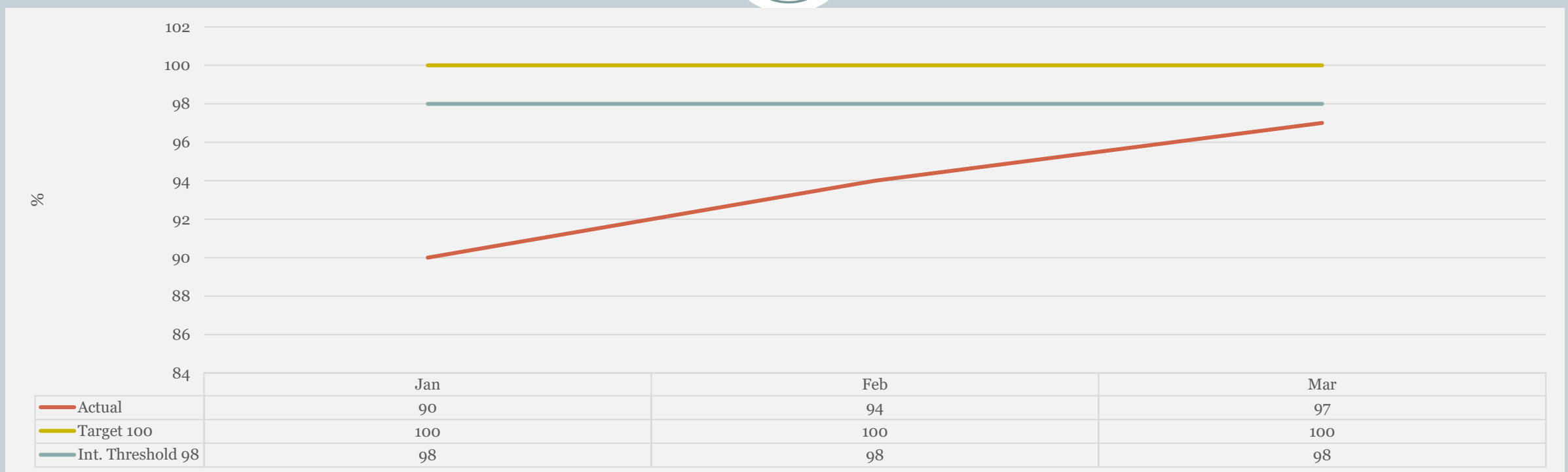


Table 24 line chart to shows the trend to compare are months of the quarter against the set, approved target (100%), and the internal threshold (98%).

Causes of Noncompliance

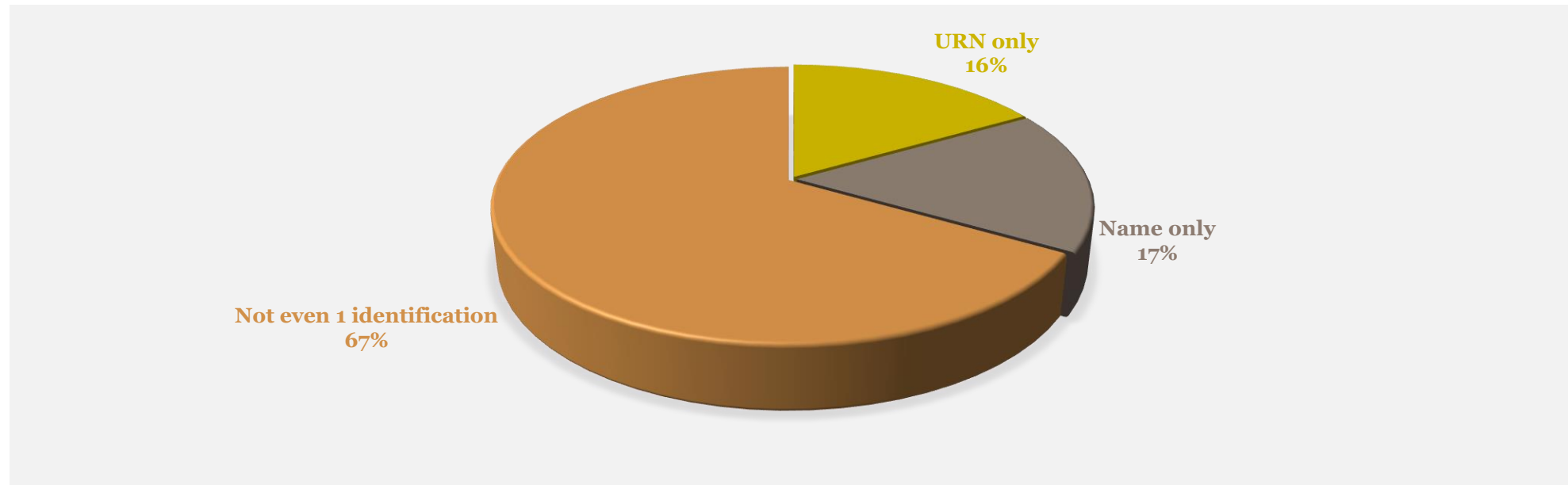
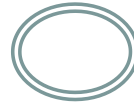
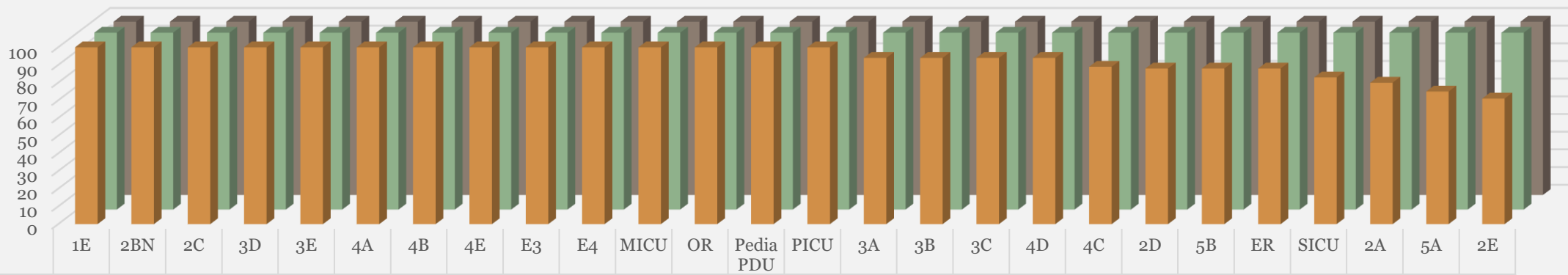


Table 25 highest cause of incompliance is “not even 1 identification with 67%, 17% for name only & URN only 16% . However, all three causes are equally important in preventing misidentification during administration of medications and treatments besides the 5 R’s of Medication Administration.

Unit Distribution



Actual	100	100	100	100	100	100	100	100	100	100	100	100	100	100	94	94	94	94	89	88	88	88	83	80	75	71
Target100	100	100	100	100	100	100	100	100	100	100	100	100	100	100	100	100	100	100	100	100	100	100	100	100	100	100
Int. Threshold 98	98	98	98	98	98	98	98	98	98	98	98	98	98	98	98	98	98	98	98	98	98	98	98	98	98	98

Table 26 overall unit specific compliance for Q1 2026 showing 14 (54%) of the total nursing units reached the target and the internal threshold. The remaining 46% (12) units are challenged to reach either the internal threshold & or the target.

% Nursing Compliance to
Patient Identification
during Medication
Administration(Process)

Actual: 93.7%

Target: 100%

Internal threshold: 98%

Fla: N/D X 100 wherein

Numerator: Number of nurses
observed compliant with patient
identification policy during
medication preparation

Denominator: Total number of
nurses observed

Multiply by 100

OVERALL COMPLIANCE

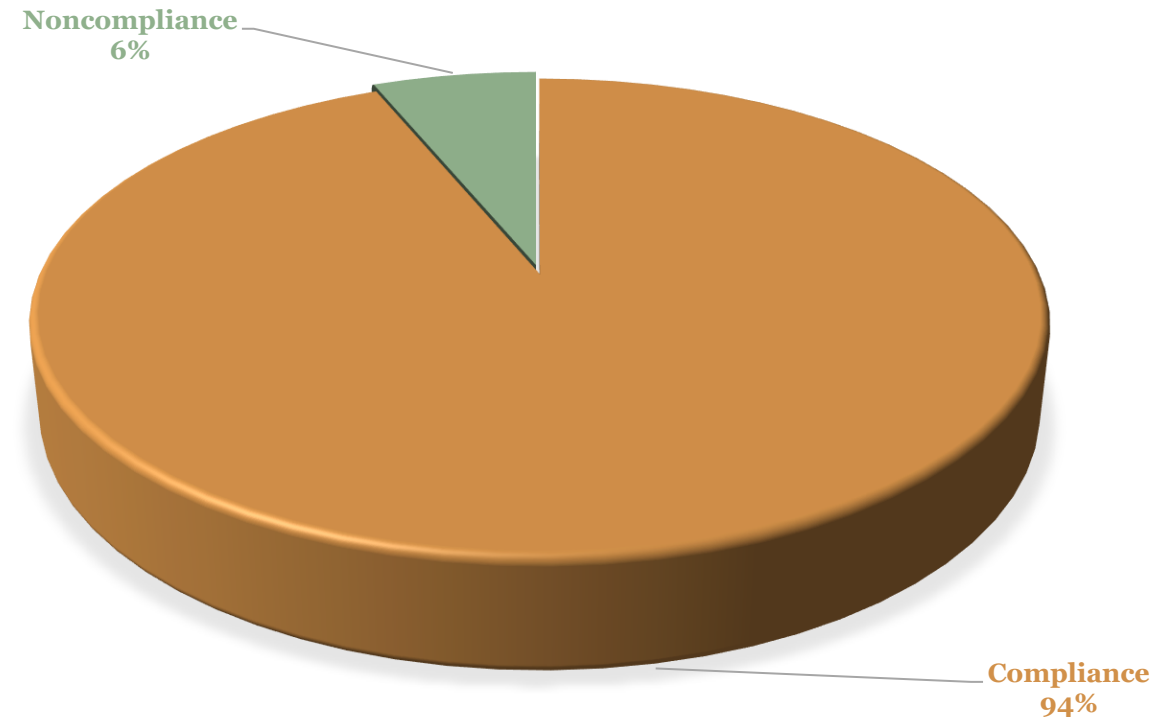


Table 27 overall compliance for 1st Quarter 2026 implies below the set target (100%) & Internal threshold (98%). Compliance to Correct Patient Identification (IPSG 1)- ADM Gen 01-035 should always be 100% in all aspects of care.

% Compliance to Patient Identification during Medication Administration

Quarter Distribution

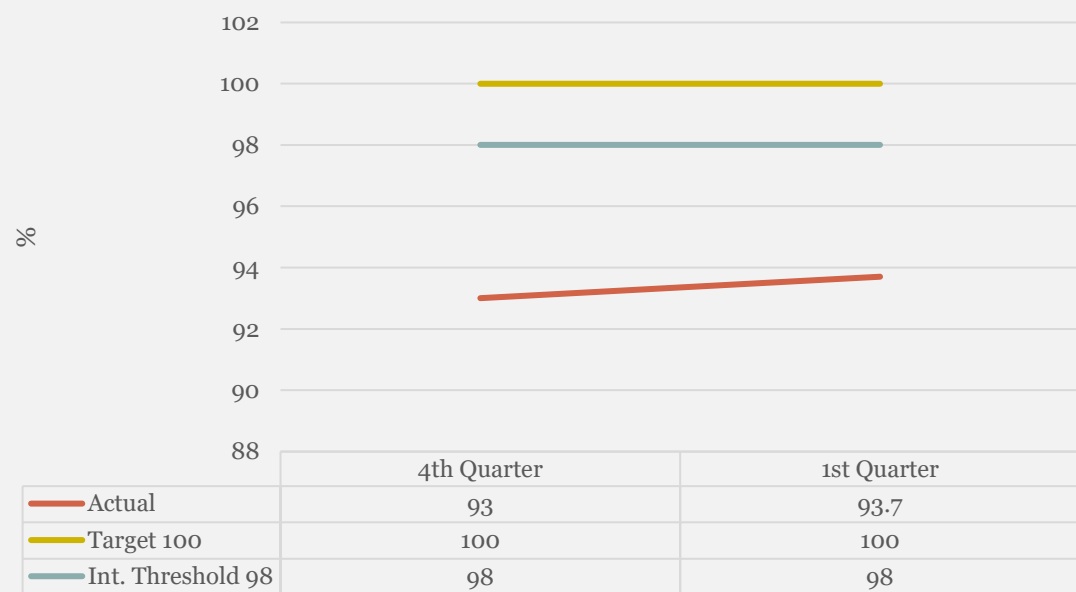


Table 28 comparison of department's performance by quarter indicating both quarters (4th [2025] & 1st [2026]) are below the set and approved target (100%) and the internal threshold (98%).

Monthly Distribution

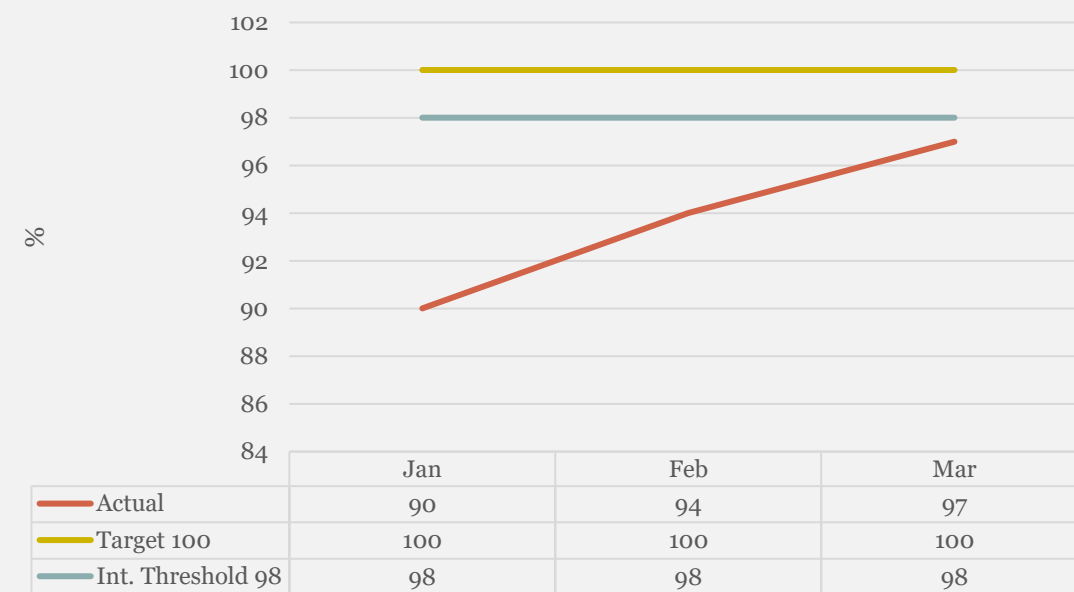


Table 29 trend the overall performance by month indicating all months are below the target (100%) and the internal threshold (98%).

Causes of Noncompliance

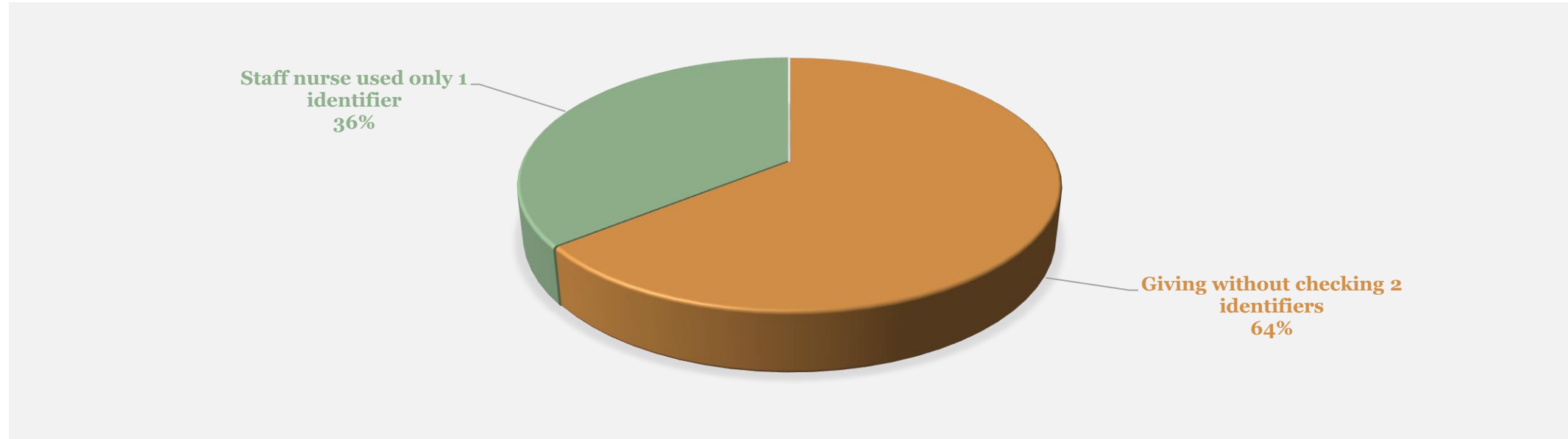
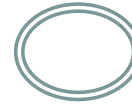


Table 30 The graph clearly identifies that our biggest threat is behavioral shortcutting. Since nearly two-thirds of staff skip the verification process entirely, our focus must shift from 'reminders' to 'active behavioral retraining.' We are seeing a direct link between staff overconfidence and a breakdown in our primary safety defense. *World Health Organization (WHO). Patient Safety Solutions: Patient Identification.*

Unit Distribution

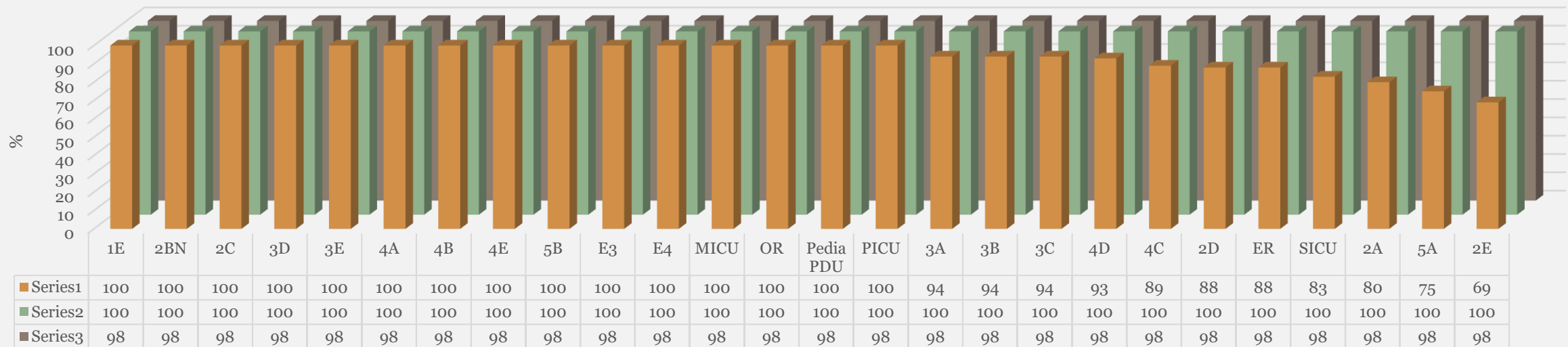


Table 31 comparison of units' performance . Additionally, comparing their own performance against themselves will challenge, encourage, and inspire them to perform beyond the set target. For those with good performance, the need to sustain is important. On the other hand, the need to reemphasize for those units with performance below the target.

Medication Related Variances (Process/Outcome)

A multidisciplinary KPI but absolutely a nurse sensitive indicator that needs to be monitored.

Internal Threshold: Establishing Baseline

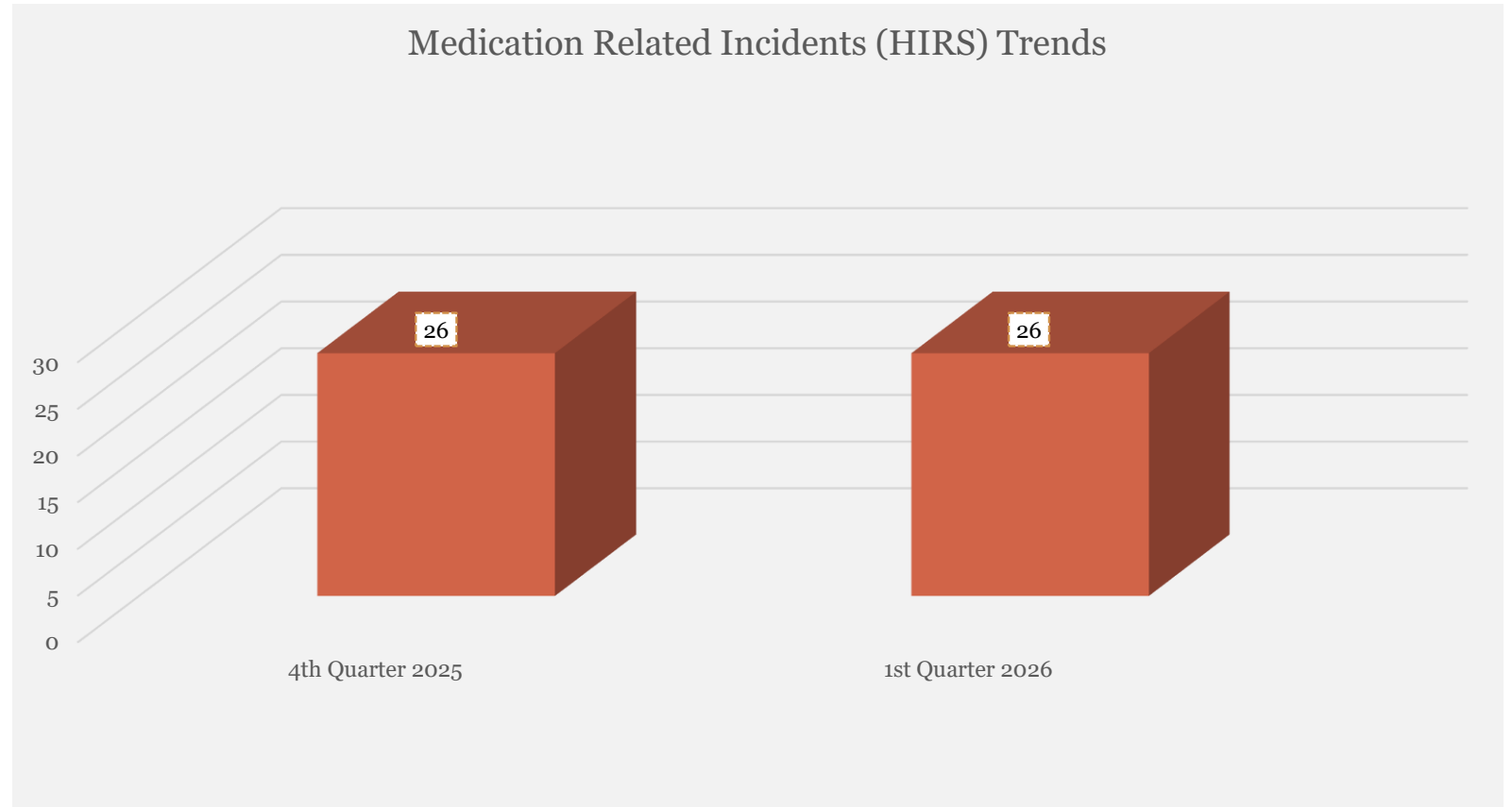


Table 32 comparison between 4th Quarter 2025 and 1st Quarter 2026 showing both reported on Medication Variances are the same.

Monthly Distribution

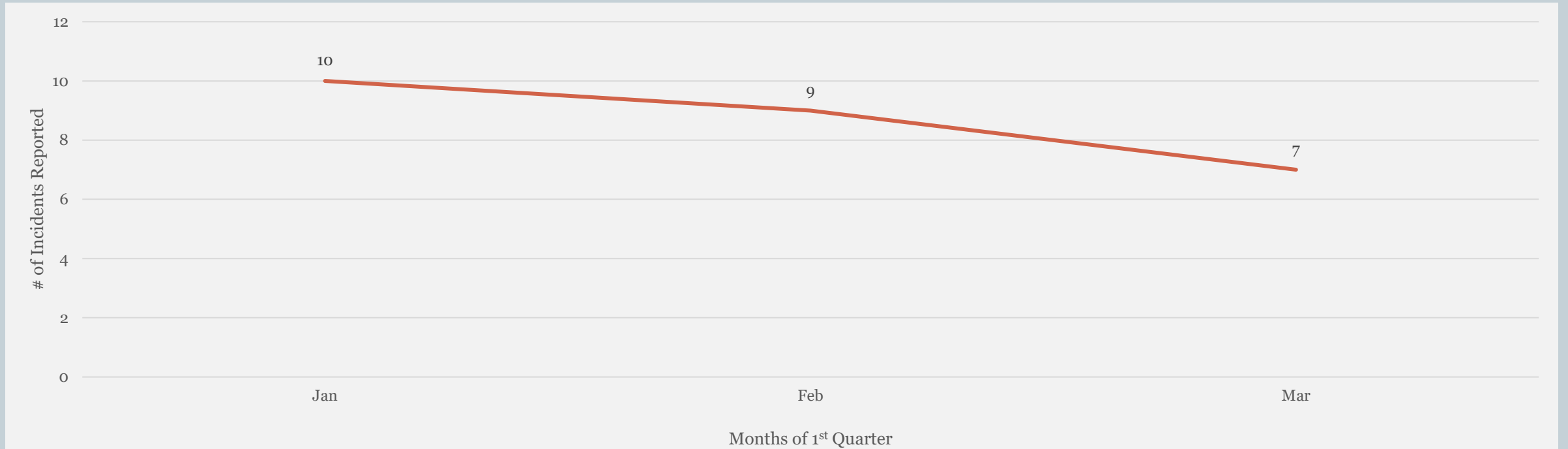


Table 33 represents number of reported medication variances by month indicating highest on the month of January garnering 38%. Focus should be done to encourage all units to report both actual & near misses. Moreover, develop mechanisms for the engagement and empowerment of patients to safely manage their own medications, WHO Global Patient Safety Action Plan 2021–2030: [patientsafety@who.int/patient safety-](mailto:patientsafety@who.int/patient%20safety)

Medication Related Variances

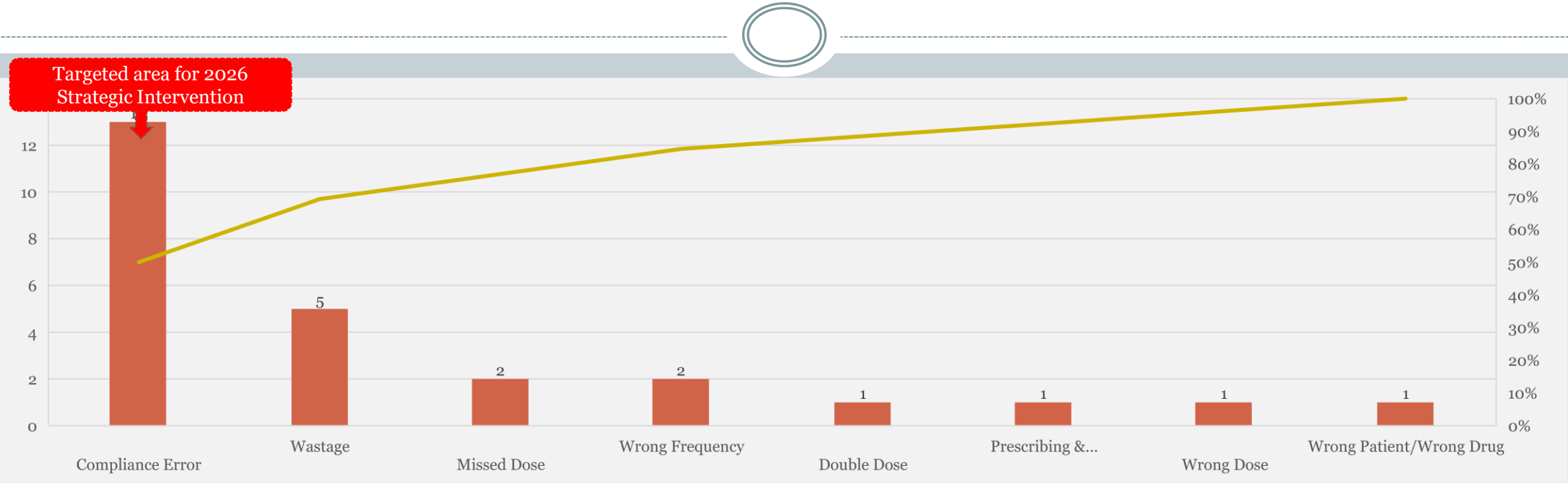


Table 34 the bar graph represents the actual medication related variances reported by all nursing units indicating 50 % falls under compliance error (i.e., in compliance to P/Ps related to medication administration) and wastage 19% respectively.

"Our 2025 baseline confirms that 50% of incidents are compliance-based; our 2026 strategy targets this 'Vital Few' through Closed-Loop HIS technology and Acuity-Based Care Models."

Medication Variances



Medication Related Incidents

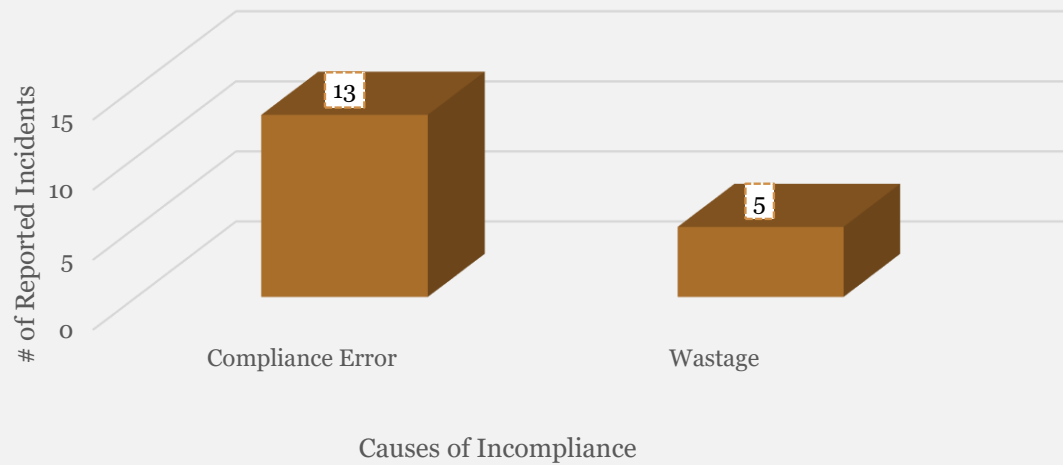


Table 35 graph represents the reported medication related incidents

Medication Error

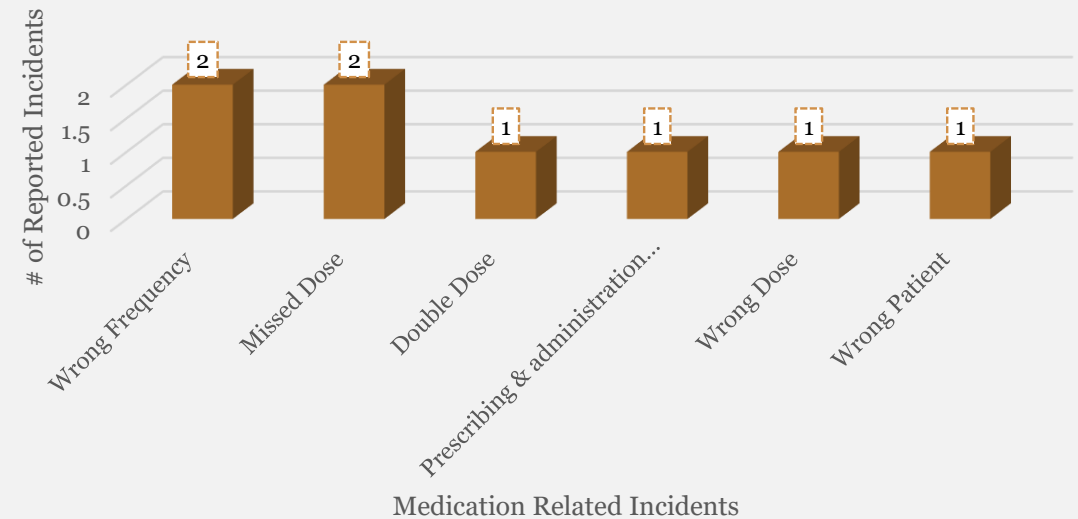


Table 36 total number of reported medication errors from 1st quarter of 2026 generated from nursing units.

Unit Distribution

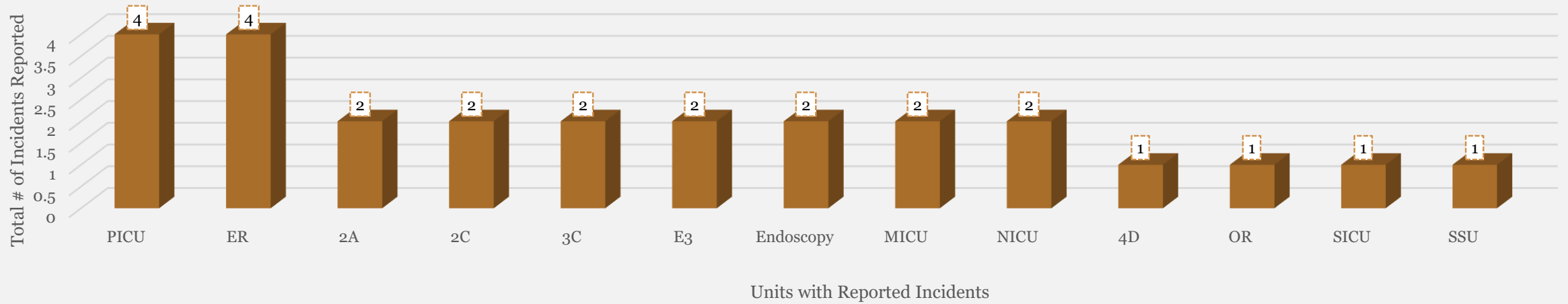


Table 37 above shows the distribution of medication variances reported by all nursing units.

Action Taken/Barriers/Next Steps:



Problem Statement:

"Between January and March, the facility recorded 26 clinical incidents, with a significant concentration (50%) categorized as 'Compliance Errors.' Despite a 30% overall reduction in incident volume during the quarter, high-acuity units—specifically the ER and PICU—remain disproportionate hotspots, accounting for 31% of all occurrences. This suggests a systemic gap between standard operating protocols and the workflow demands of high-intensity clinical environments."

Finalized Analysis:

1. The Core Issue: Protocol-Process Gap

The data indicates that the "Compliance Error" is not an isolated human failure but likely a systemic "workaround" issue. In high-pressure areas like the ER and PICU, staff may be bypassing steps to maintain patient throughput, which is evidenced by these units having double the error rate of others (e.g., 4D or the OR).

2. The Secondary Concern: Resource Management

Wastage accounts for nearly **20%** of the data. RCA documented if this wastage is "forced" (e.g., single-dose vials being used for pediatric doses in the PICU) or "procedural" (e.g., meds being drawn but not administered due to clinical changes) or again it's human factor.

3. Evidence-Based Practice (EBP) Recommendations

A. Human Factors Engineering: Unintentional wastage, overriding medications (clinical no entry), and compliance to patient identification

B. Unit-Specific "Safety Huddles": Implement a 5-minute shift-change huddle in the ER and PICU specifically focused on the "Top 2" errors from the previous month. Moreover, the focus on no distraction during medication preparation, and most importantly, reiteration of using **the 2 identifiers** in addition to **the five (%) Rs** of medication preparation & administration.

C. Just Culture Framework: Use this data to reinforce a "Just Culture," where the focus remains on fixing the system defects in the PICU/ER rather than punitive measures for the staff involved. Initiated during RCAs considering a fair and no blame way of investigation.

% Nursing Compliance to Correct Handover using SBAR tool (Process)

Actual: 79%

Target: 95%

Internal Threshold: 98%

Fla: N/D X 100 wherein

Numerator: Number of nurse observed compliant with (SBAR) Policy

Denominator: Number of nurses observed

Multiply by 100

Noncompliance
21%

OVERALL COMPLIANCE

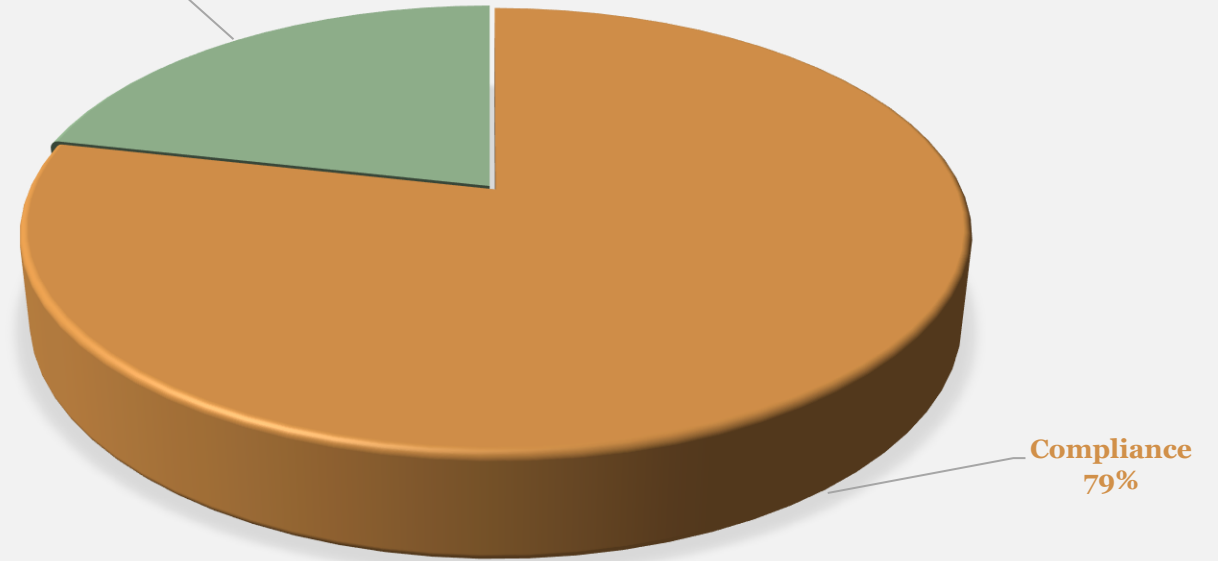


Table 38 graph shows the overall compliance of the department indicating below the target & internal threshold. A need to strengthen awareness of enhanced communication in the transfer of care from one staff to another. Poor communication can result in various negative outcomes, such as decreased adherence to treatment, patients' dissatisfaction and inefficient use of resources, lack of continuity of care, and medical errors.(nih.com).

Quarter Distribution



Table 39 represents the comparison between the 4th quarter 2025 and the 1st quarter 2026 indicating a slight increased but still both are below the target (95%) and the internal threshold 98%.

% Compliance to Correct Handover utilizing SBAR Tool

Monthly Distribution

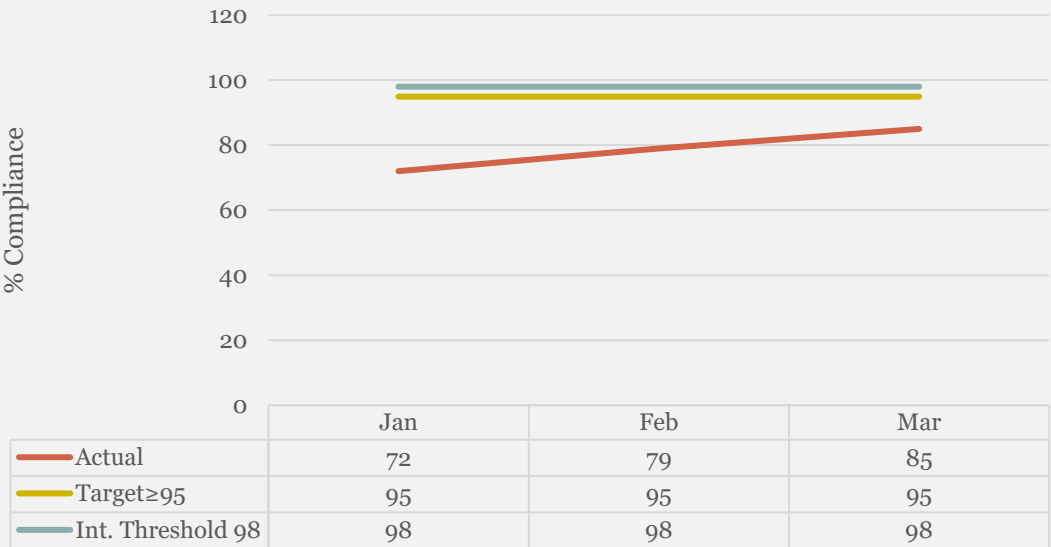


Table 40 all months of the 1st quarter 2026 on compliance showing below the target (95%) and internal threshold (98%).

Causes of Incompliance

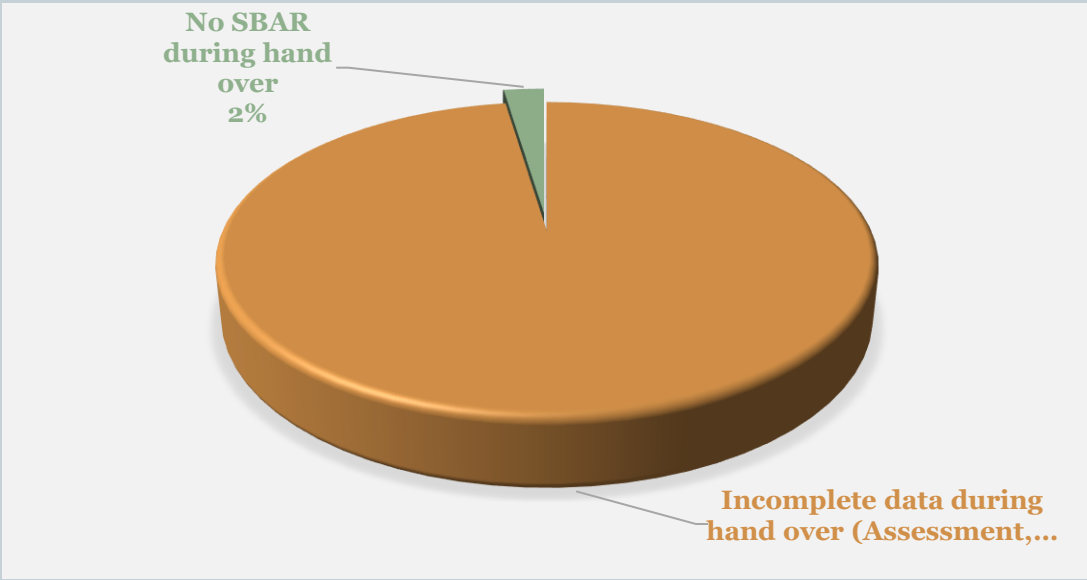


Table 41 the graph represents the causes of noncompliance. Both causes impact the appropriateness of handover in improving communication thereby prevention medical errors.

Unit Distribution

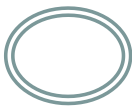


Table 42 represents unit specific performance showing that only 7% of the total nursing units exceeds the set target (95%) and the internal threshold (98%). This implies a need to reemphasize the importance of good communication in the transfer of care and strengthening patient safety.

Action Taken/Barriers/Next Steps:



1. High-Impact Strategies for IPSG 2 Compliance

To move the needle effectively, consider these targeted adjustments to our current steps:

- Real-Time "Stop-the-Line" Audits: Instead of quarterly updates, use NQI and Unit Managers to perform "active huddles" where they observe handovers in real-time. If the electronic SBAR isn't being used, they provide immediate, on-the-spot coaching to correct the behavior.
- Electronic Workflow Optimization: Work with the IPSG Chapter to ensure the electronic SBAR isn't just a form but a forcing function in the EHR. For example, making certain SBAR fields mandatory before a nursing note can be finalized for shift change.
- Focus on High-Risk Transitions: Compliance often drops during high-stress times. Focus initial reinforcement on Night-to-Morning handovers and Emergency Department transfers, where communication breakdowns are most frequent.
- "Error-Wisdom" Training: Shift education from "how to use the tool" to "what happens when we don't." Use real, de-identified case studies of "near misses" in your facility caused by poor communication to build a sense of urgency.

2. Addressing the 79% vs. 98% Gap A 79% rate often indicates that the tool is seen as a "burden" rather than a "helper."

Current Barrier	Strategic Response
Human Factors (Hurry)	Implement "Quiet Zones" for handover to reduce distractions and the "human factor" of rushing.
Staffing Ratios	Nursing administration should prioritize staggered break times or dedicated "handover blocks" where incoming staff are not immediately assigned new tasks.
Tool Friction	Reinforce to all to ensure the electronic SBAR auto-populates basic data (vitals, meds) so nurses only have to manually input the Assessment and Recommendation.

3. Visual Performance Tracking - Weekly "SBAR Champion": Recognize a staff member or unit that hit 100% compliance for the week; Trend Lines: Show the gap closing from 79% toward 98% visually so staff can see the direct impact of their teamwork. National Institute of Health (NIH)



Pediatric IV Infiltration(Outcome)

Actual: 1.24

Internal Threshold:
Establishing Baseline

Fla: $N/D \times 100$ wherein

Numerator: Number of
patients with Infiltrated IV
Cannula

Denominator: Total Number
of patients in the Pediatric
Units (4A,4B, NICU,PICU)

Multiply by 100

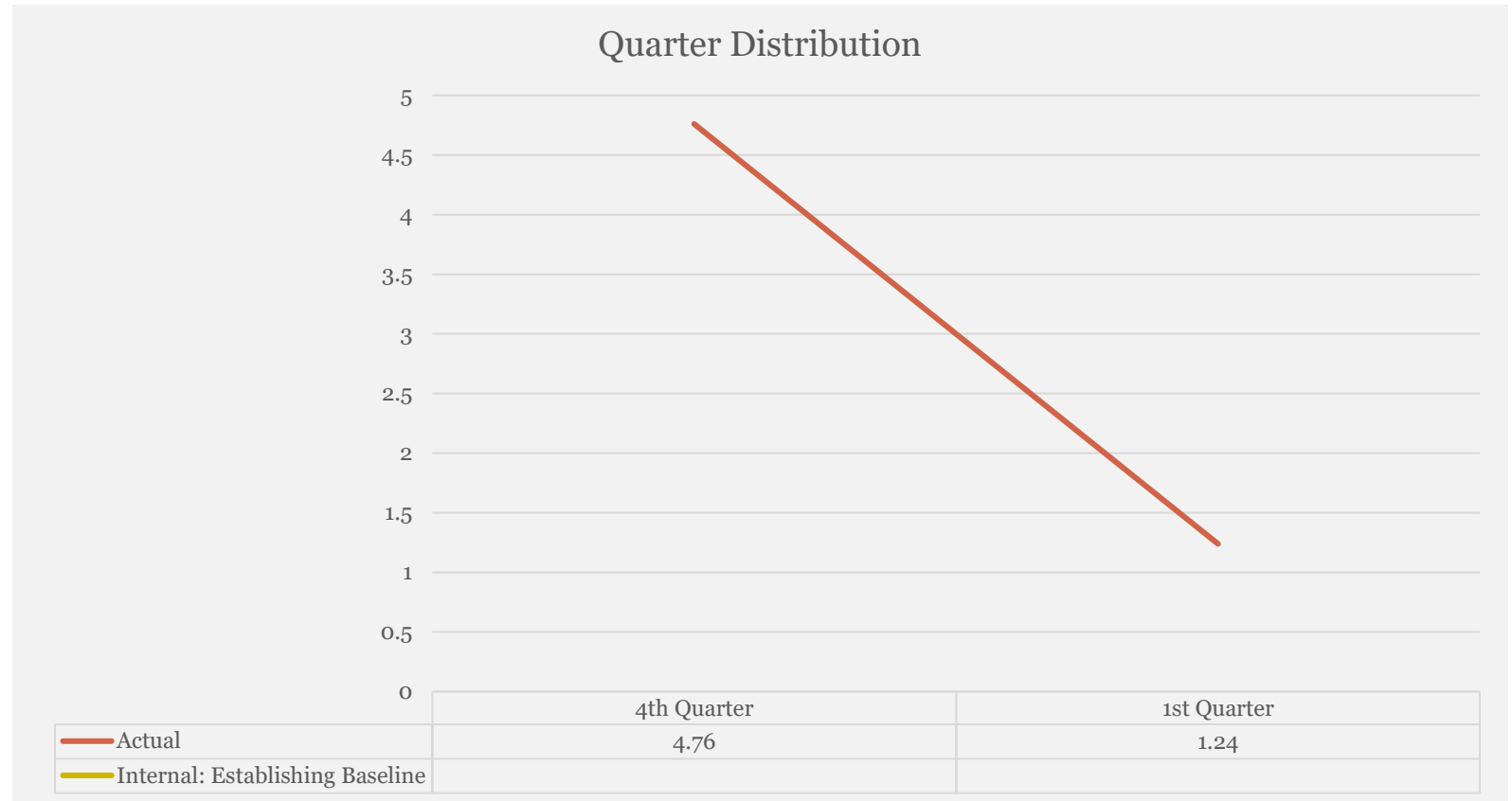


Table 43 graph shows all quarters of both years (2025/2026). Shifting from the identified, most appropriate denominator, internal threshold will be utilizing 2025 as the baseline.

Monthly Distribution

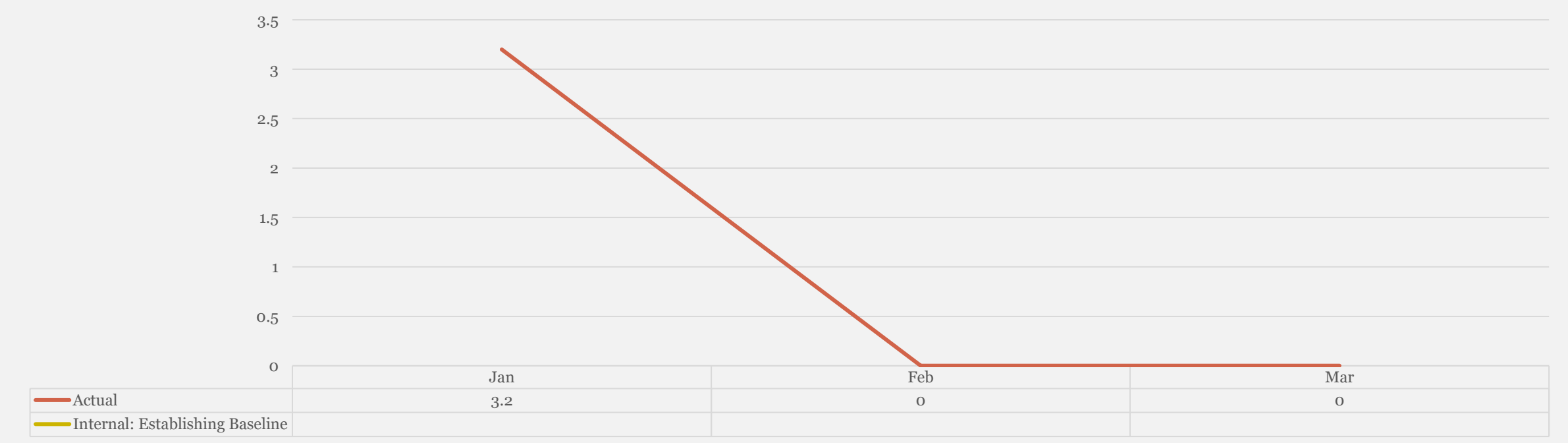


Table 44 a line graph that trends from Jan-Mar 2026. Both Feb and Mar has no reported PIV.



Adult IV Infiltration(Outcome)

Actual 0.82

Internal Threshold: Establishing
Baseline

Fla: N/D X 1000 wherein

Numerator: Number of
patients with Infiltrated IV
Cannula

Denominator: Total Number
of adult patients with IV
Peripheral Lines

Multiply by 1000

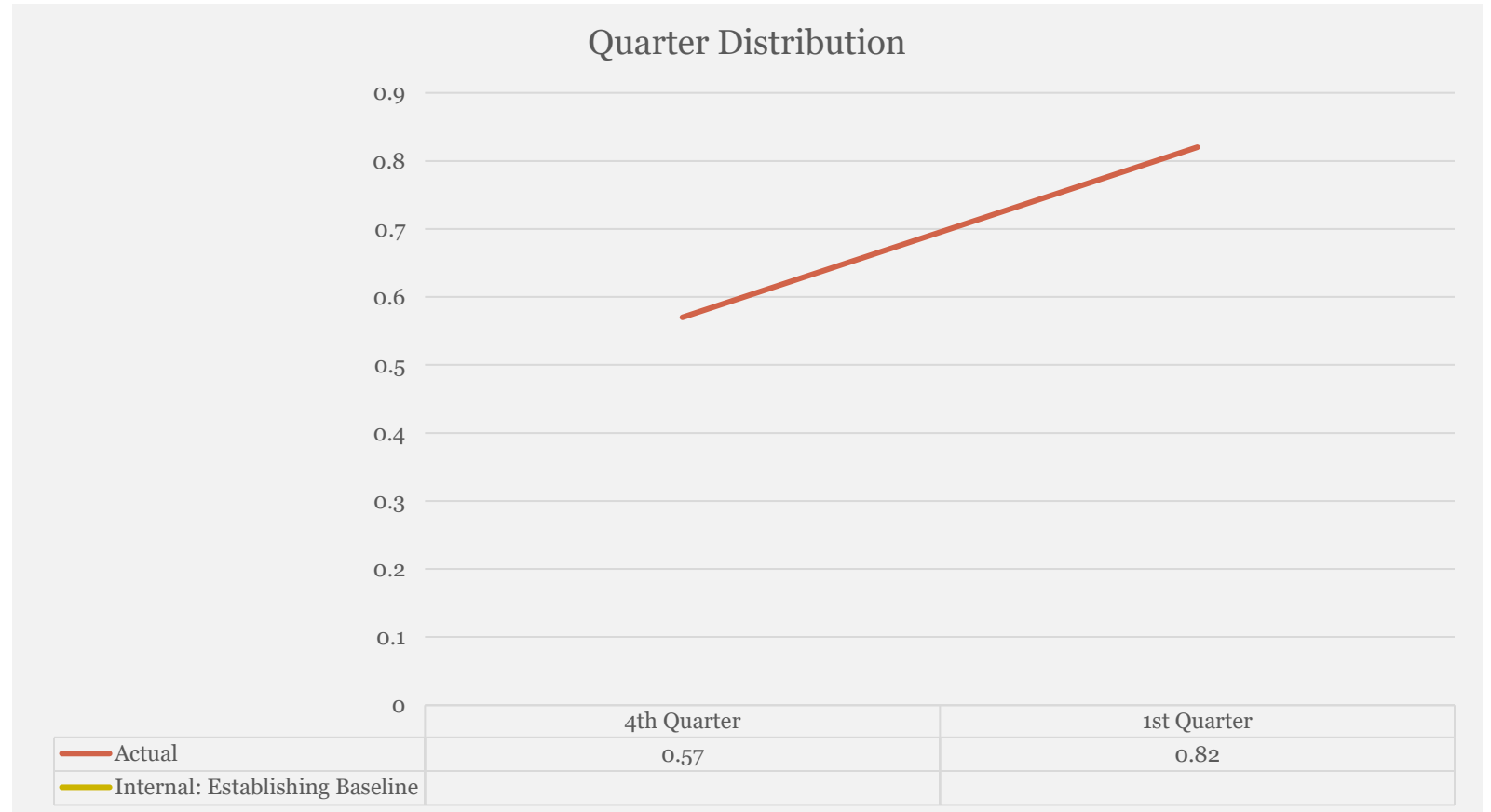


Table 45 graph shows peripheral IV infiltration rate among adult patients. A new initiative- KPI launched to address issues on IV lines (difficult vein access/complications). For the internal threshold, the 2025 data will be utilized as baseline.

Monthly Distribution

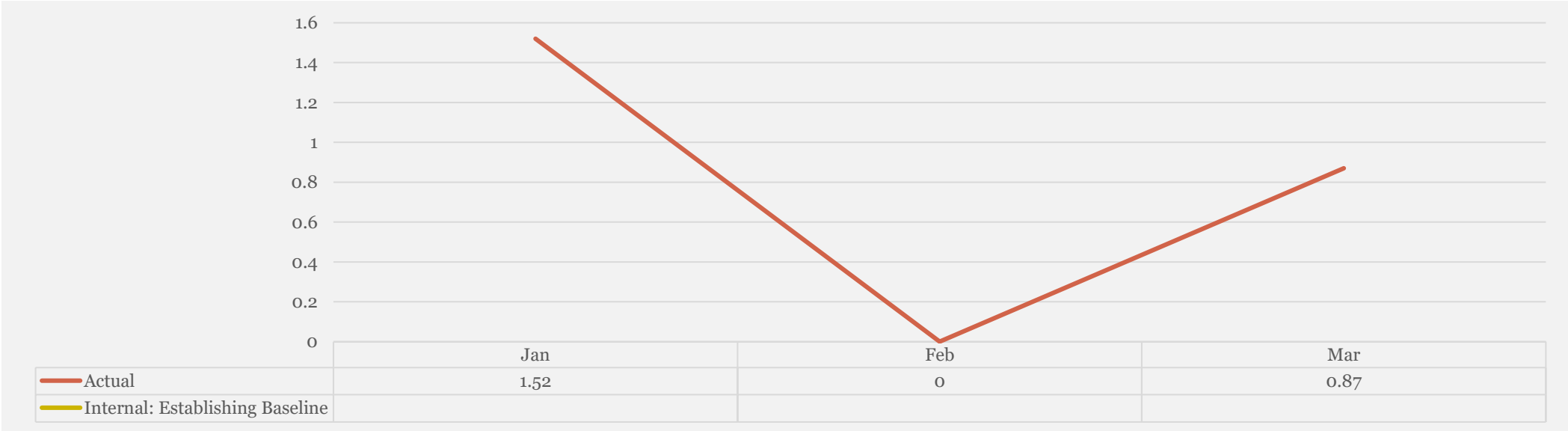
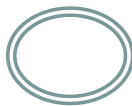


Table 46 a line graph that trends from Jan-Mar 2026. February has no reported PIV.



Extravasation following Medication Administration Among Pediatric Patients (Outcome)

Actual: 2.47

Internal Threshold: Establishing Baseline

Fla: N/D X 1000 wherein

Numerator: Number of patients with Infiltrated IV Cannula

Denominator: Total Number of patients with IV Peripheral Lines

Multiply by 1000

Extravasation following Medication Administration among Pediatric Patients

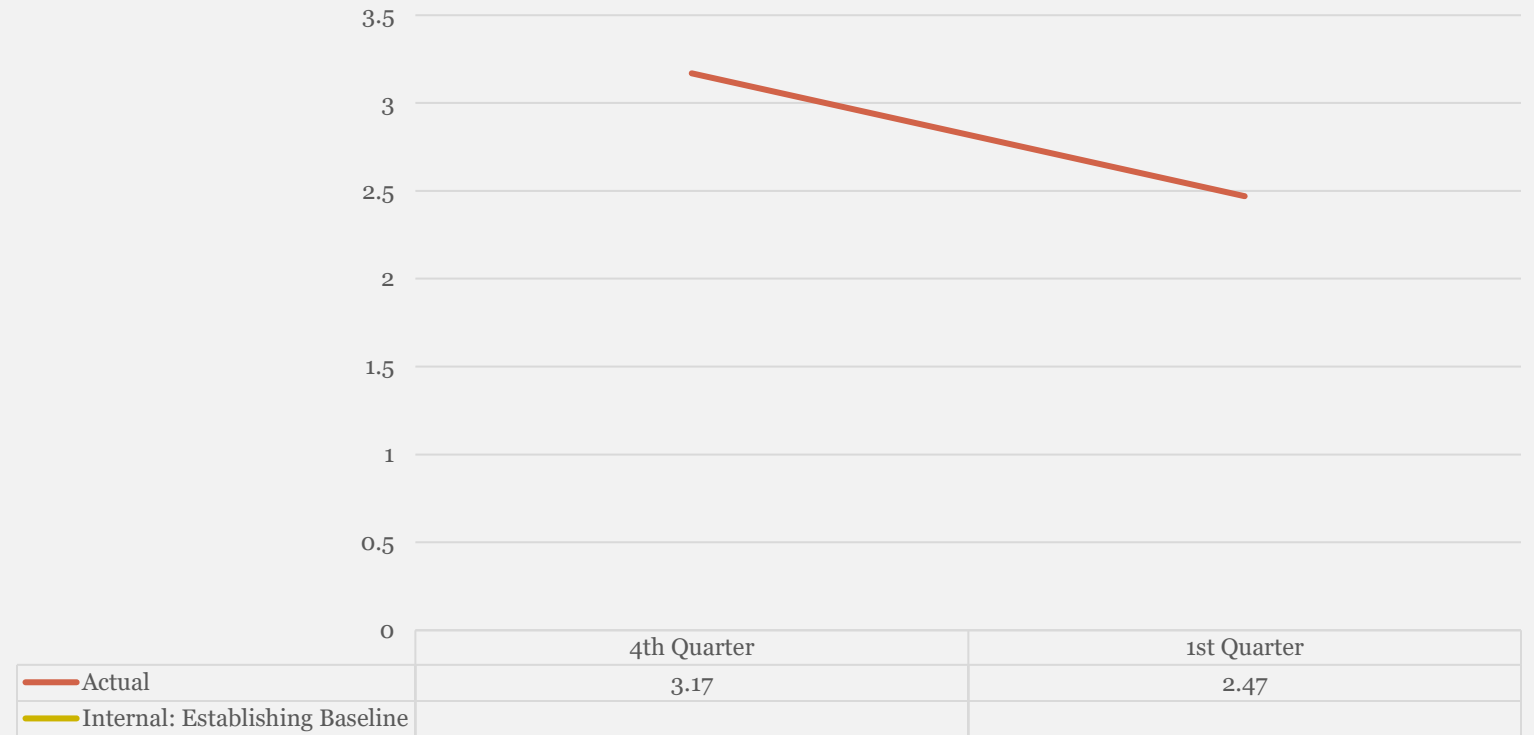


Table 47 graph shows reported total number of extravasated lines among pediatric patients comparing both the 4th quarter 2025 & 1st quarter 2026 indicating a decreased.



Extravasation following Medication Administration Among Adult Patients (Outcome)

Actual: 0.95

Internal Threshold: Establishing Baseline

Fla: N/D X 1000 wherein

Numerator: Number of patients with Infiltrated IV Cannula

Denominator: Total Number of patients with IV Peripheral Lines

Multiply by 1000

Extravasation following Medication Administration among Adult Patients



Table 48 graph shows reported total number of extravasated lines among adult patients comparing both the 4th quarter 2025 & 1st quarter 2026 indicating a decreased.

Monthly Distribution

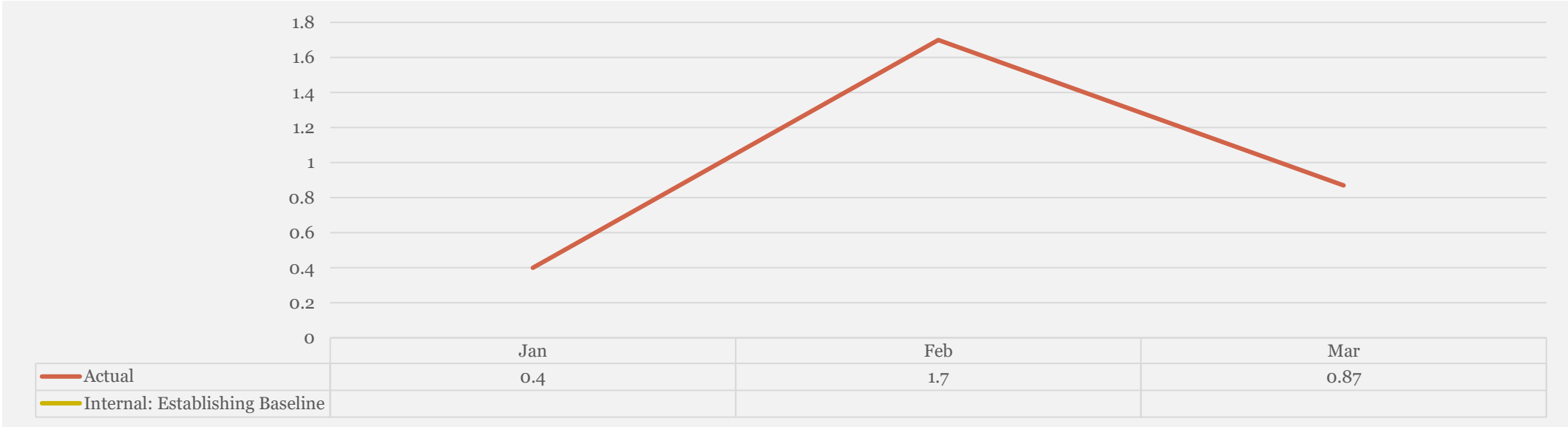


Table 49 a line graph that trends from Jan-Mar 2026.

Action Taken/Barriers/Next Steps:



Active Monitoring & Assessment: Nursing Quality Improvement (NQI) officers in collaboration with the unit managers (Clinical Supervisors/HN/CNs/Designees) are conducting continuous checks on dressing, labeling, and site integrity. A core focus remains on hourly rounding and early assessment of peripheral IV sites.

Education & Differentiation: Strengthening unit-wide awareness to accurately distinguish between IV infiltration (leakage of non-vesicant solution) and Extravasation (leakage of vesicant or irritant solution).

Protocol Development: Implementing Evidence-Based Practice protocols, such as those cited by Desarno et al. (2018), to standardize care and reduce iatrogenic injuries.

Incident Reporting: Promoting the use of the Hemaya Incident Reporting System (HIRS) to ensure all complications are documented and analyzed.

Specialized Teams (DIVA): Finalizing the creation of a multidisciplinary Difficult Intravenous Access (DIVA) team and establishing guidelines for patients who are hard to cannulate.

Pediatric PI Project: Completing the final phase of the Pediatric IV Infiltration project, which includes reviewing nursing policies, activating the DIVA team, and ensuring supply availability.

Technological Integration: Moving forward with the "IV Watch" pilot project, a collaborative effort between Pharmacy and Nursing designed for the early detection of infiltration and phlebitis (pending final approval)- full implementation.



Nursing Staff Ratio in Critical Care Units (Structure)

External Benchmark: 1.2 (MOH)

Fla: N/D X 100 wherein

Numerator: Number of Nursing Staff at Critical units (MICU,SICU, CCU, PICU, NICU)

Denominator: Total Number of occupied bed in the unit
Multiply by 100

Nursing Staff Ratio in Critical Units

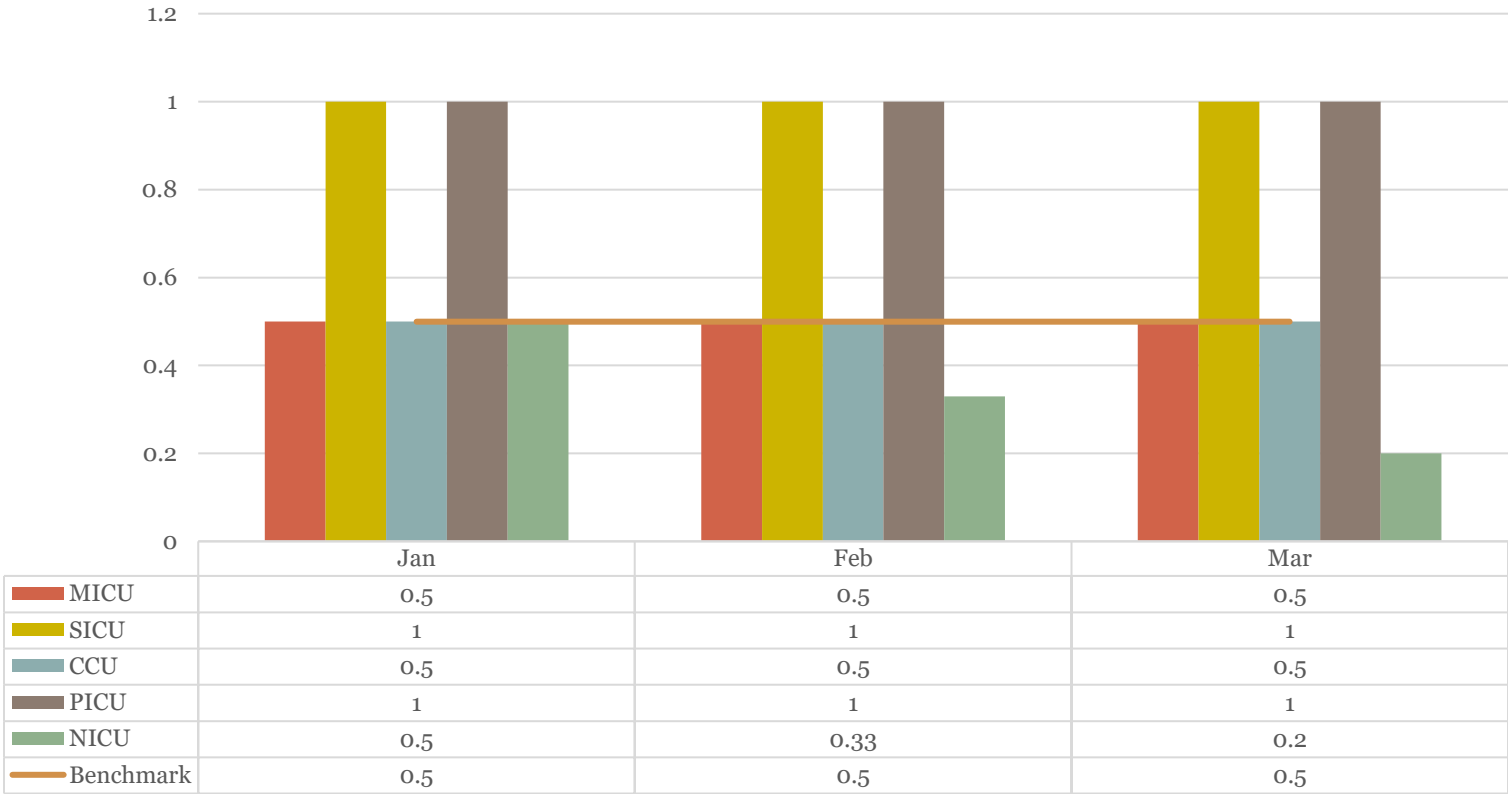


Table 50 graph represents monthly performance compared with the benchmark for the whole months of the 1st quarter 2026.

Action Taken/Barriers/Next Steps:



Action Taken:

Benchmarking: Established a consistent 0.5 (1:2) monitoring benchmark across all critical care areas to align with JCI COP (Care of Patients) standards.

Resource Reallocation: Successfully optimized NICU staffing from 0.5 to 0.2 through strategic shift management.

Vigilance Training: Initiated "Active Surveillance" education (Touch, Look, Compare) in high-staffed units following recent "late-catch" IV incidents.

Barriers:

Chronic Over-Extension: SICU and PICU remain consistently at 1.0 (100% over benchmark), creating a persistent "Surveillance Gap" that correlates with higher NSI risks.

Task Saturation: High patient-to-nurse ratios (1:10 on regular units and 1:1 on over-capacity ICUs) prioritize immediate clinical tasks over preventive documentation and reporting.

Next Steps:

Targeted Augmentation: Propose a "Float Support" model specifically for units exceeding the 0.5 benchmark to stabilize surveillance capacity.

Reporting Integrity Audit: Collaborate with DQS to validate "Zero" incident months in high-workload units to address potential under-reporting.

AOP/COP Alignment: Present staffing data to executive leadership as a core component of JCI 8th Edition compliance, focusing on the hospital's responsibility to provide resources for safe assessment.



% Compliance to Blood & Blood Product Policy (Process)

Actual: 85.21

Target: 100%

Int. Threshold: None Yet

FLA: D/N X 100 wherein

Denominator: Total number of compliant with blood & blood products transfusion policy

Numerator: Total number of patients on blood & blood products transfusion

Multiply by 100

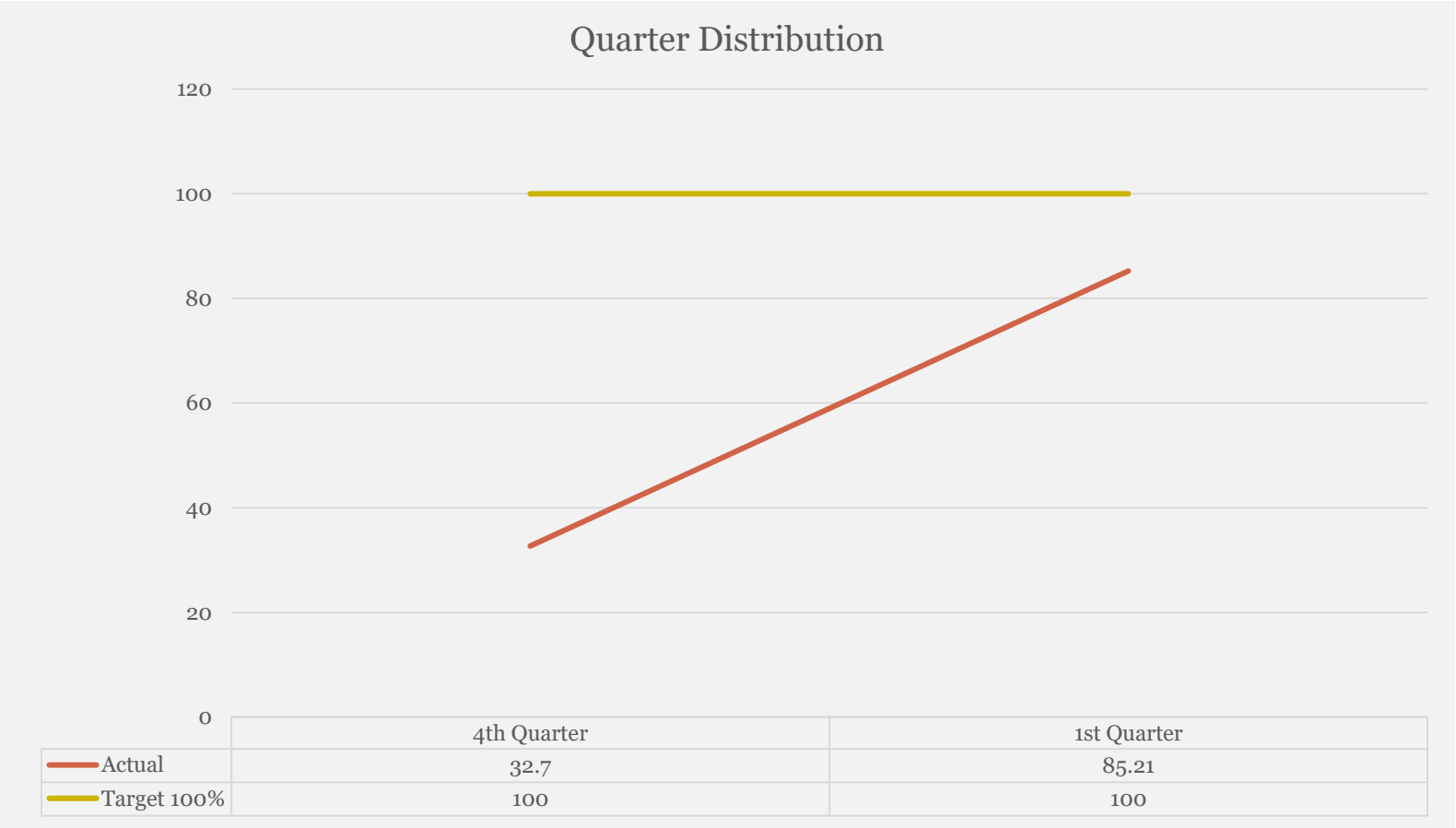


Table 51 Trends compliance between the 4th quarter (2025) and the 1st quarter (2026) indicating a significant increased.

Monthly Distribution

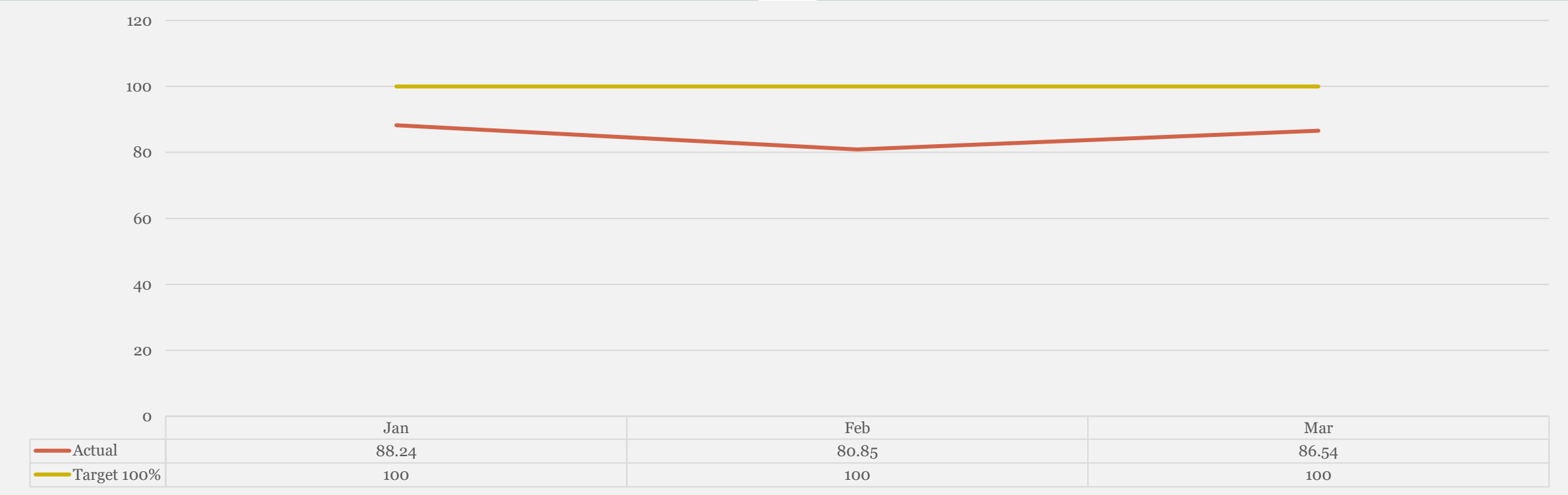


Table 52 represents the monthly performance for 2026. Trends the whole three (3) months of the 1st quarter 2026 indicating good performance, however, all three months are below the target (100%).



% Compliance on Pain Screening (Process)

Actual: 93%

Target: ≥95%

FLA: D/N X 100 wherein

Denominator: Total number of patients screened for pain

Numerator: Total number of reviewed patients record

Multiply by 100

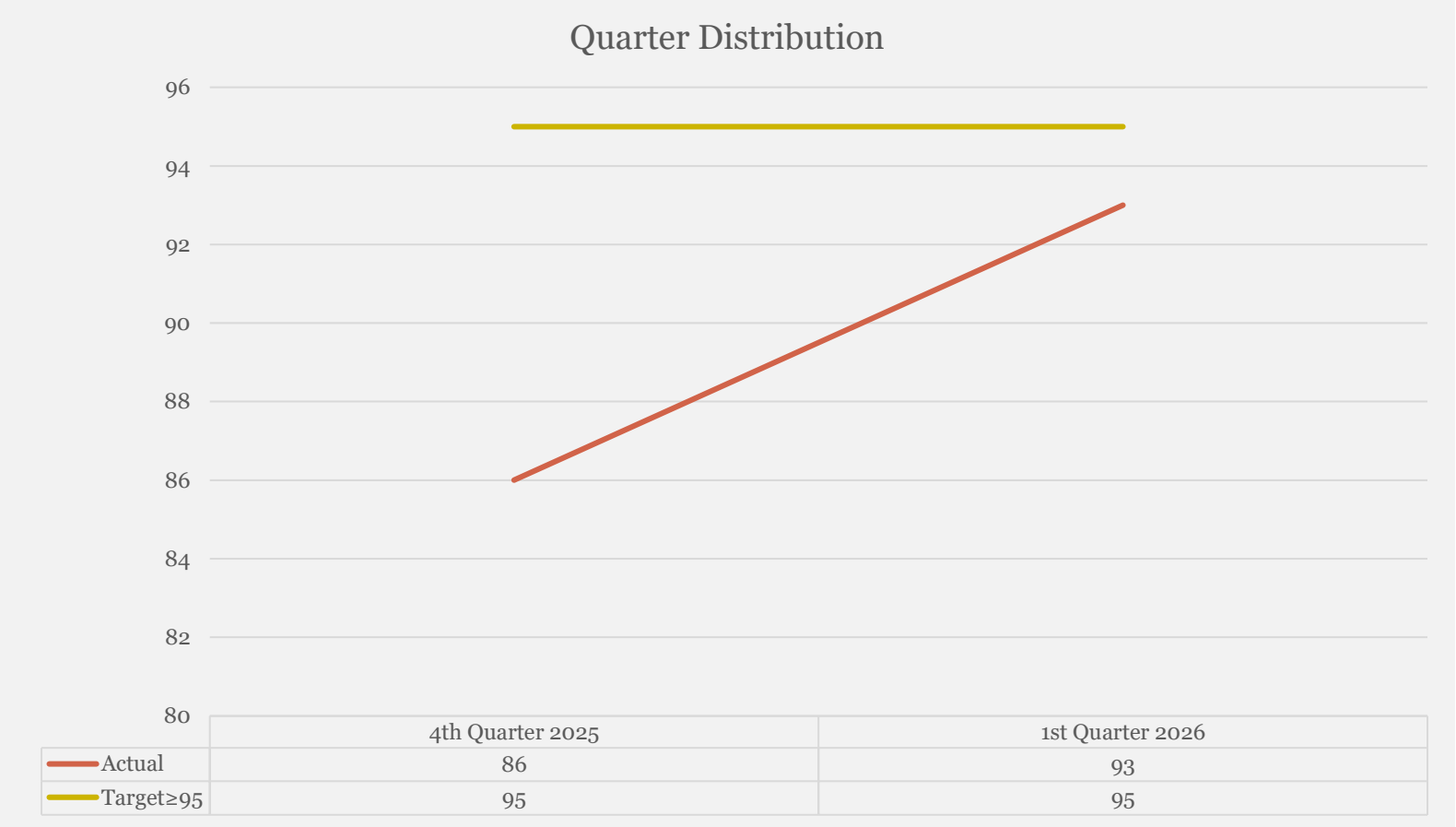


Table 53 represents the quarterly performance for the 4th quarter 2025 & the 1st quarter 2026 indicating all below the target.

Monthly Distribution

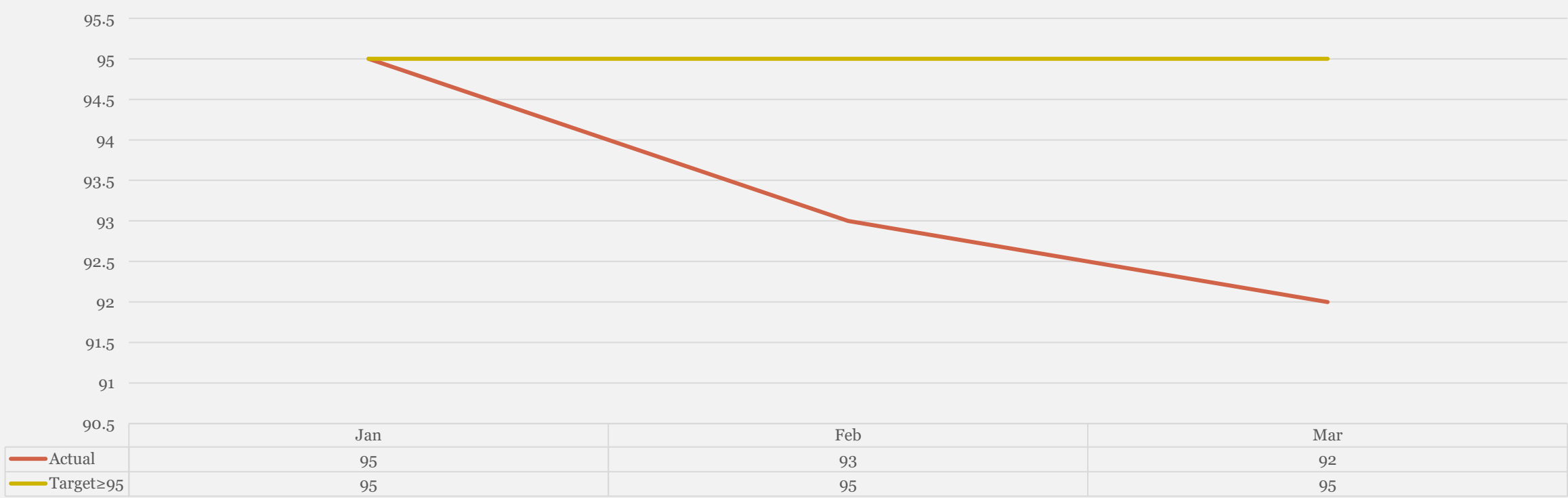


Table 54 represents the monthly compliance on Pain Screening. All months are below the target except for the month of January.

% Compliance to Pain Assessment, Intervention, & Reassessment (AIR Cycle) (Process)

Actual: 51.56

Target: ≥95%

Formula: $N/D \times 100$ wherein,

Numerator: Total Number of Patients Assessed for Pain

Denominator: Total Number of reviewed Patient’s Record multiply by 100

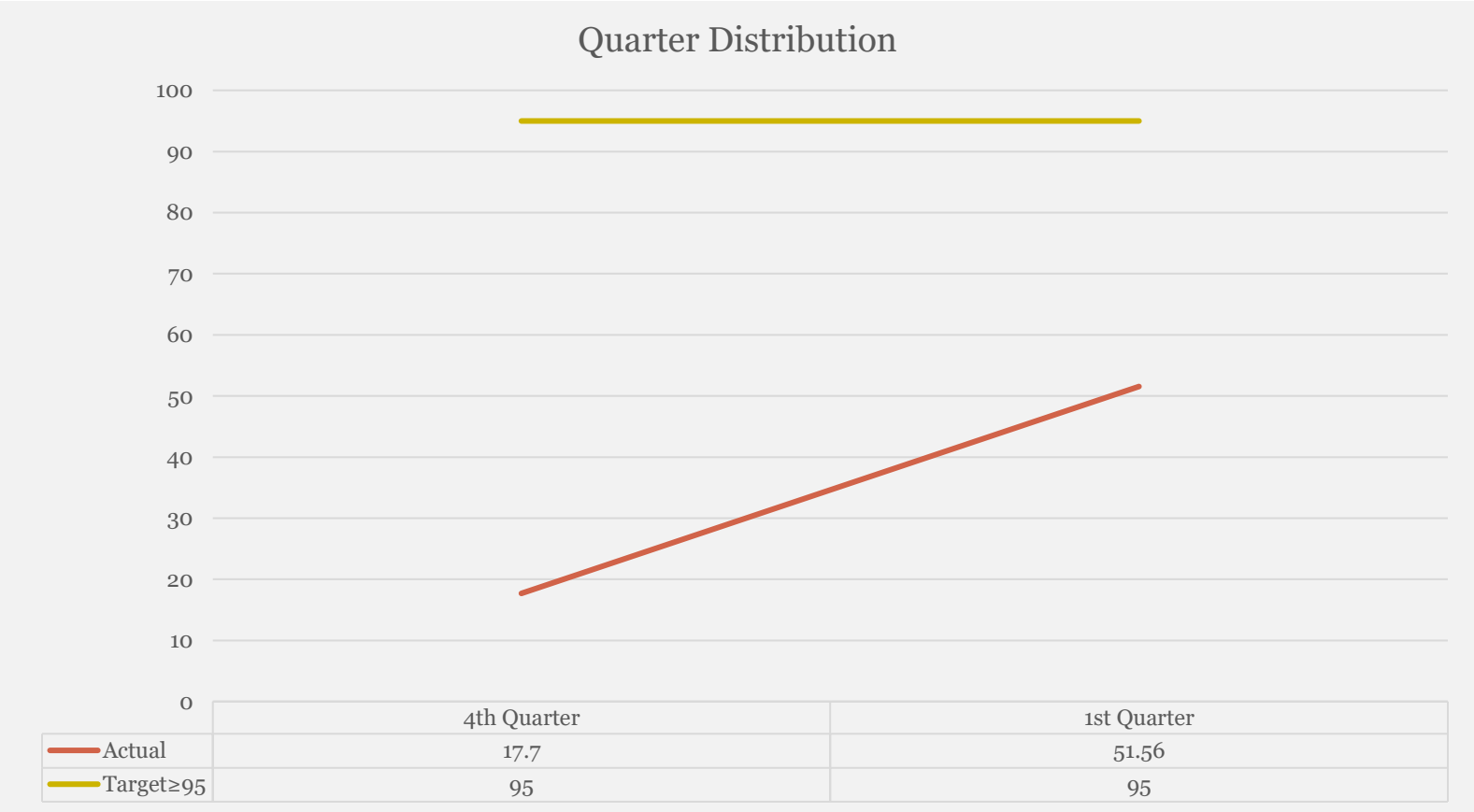


Table 55 trends the compliance on all quarters of 2025 (4th) & 2026 (1st) indicating all below the target (≥95%). A challenge for all nursing units to find strategies for the staffs in compliance to ADM Gen 01- 033 Pain Assessment & Management and to JCI standards on ACC .01.04 & AOP. 01.04 (JCI Hospital 8th Edition Manual 2025)

Monthly Distribution

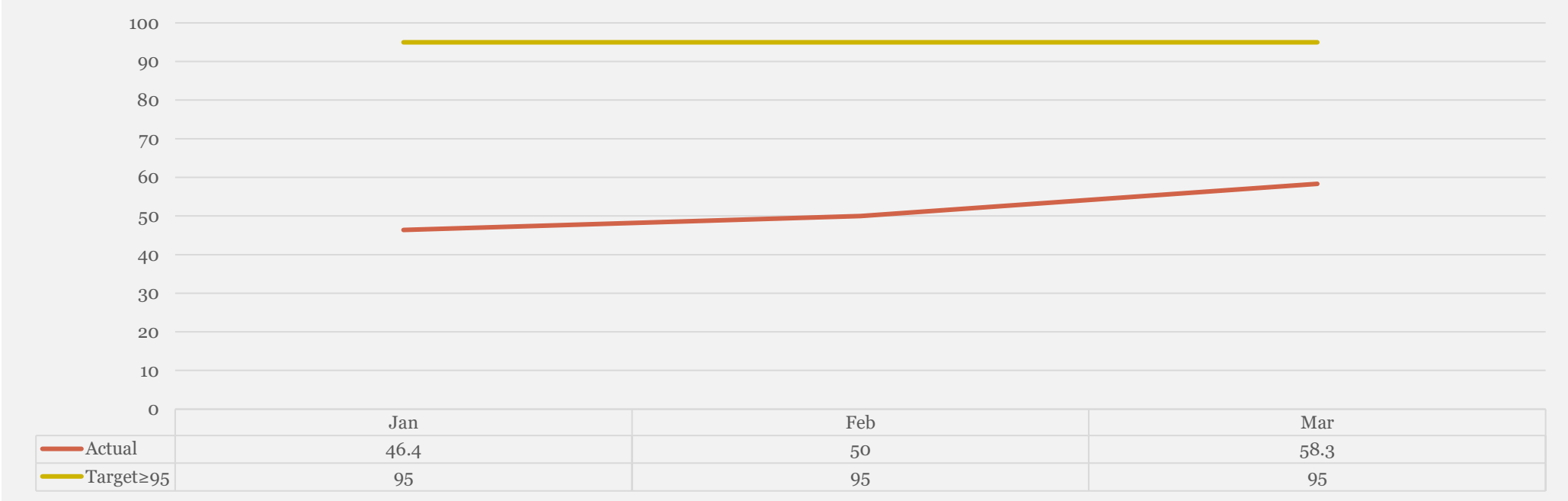


Table 57 Line chart trends compliance between months of the 1st quarter indicating all below the target.

Compliance by Phases

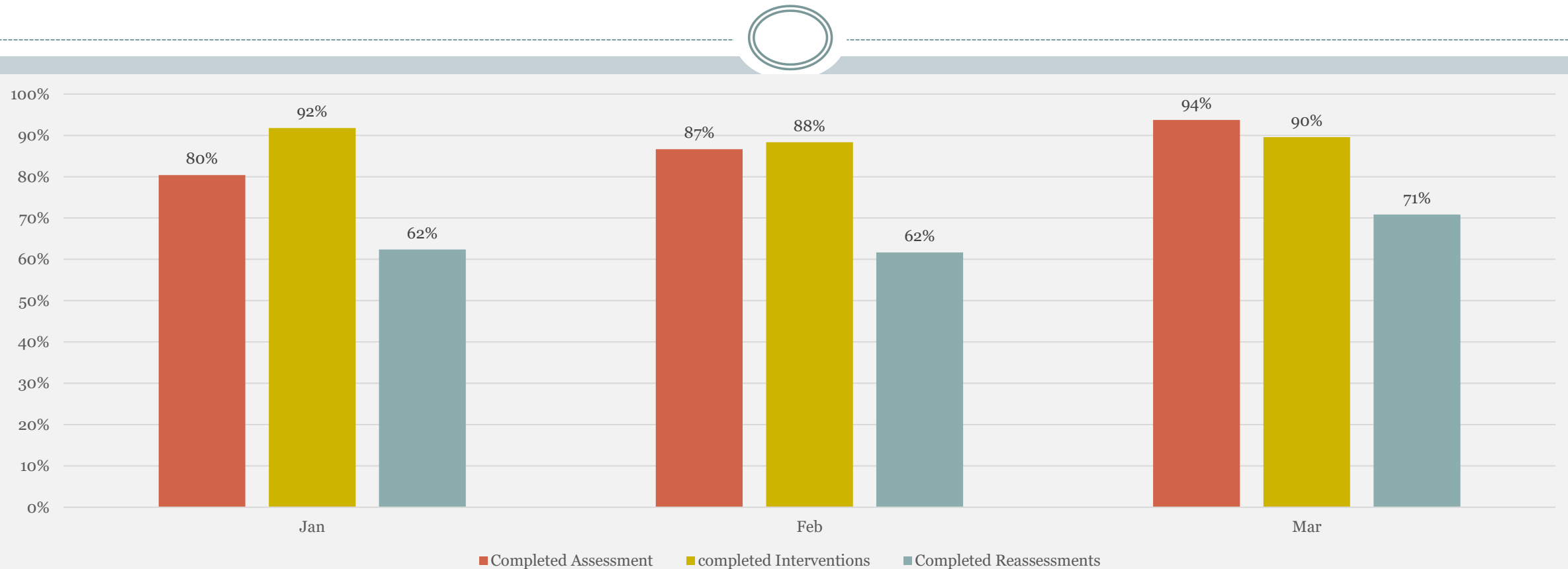


Table 58 graph shows the compliance by phases. Although all three (3) phases of the whole cycle is interrelated and equally important to complete the cycle, the purpose of this graph is to present what phase of the cycle that needs more focusing and initiate a more realistic and specified strategy.

Action Taken/Barriers/Next Steps:



1. KPI Monitoring & Target Alignment

- Action:** Conduct weekly random chart audits across all nursing units to measure the "Pain AIR Cycle Compliance" rate.
- Target:** Aim for **>95% compliance** in documentation for:
 - Screening:** Completed within **30 minutes** of patient arrival to the unit or beginning of the shift.
 - AIR Cycle:** Reassessment documented within **15-30 minutes** for IV analgesics and **30-60 minutes** for oral medications & non pharmacological.

2. Addressing the "Human Factor" in Technology Adaptation

- Action:** Deliver targeted "Micro-Learning" sessions that focus on the *why* behind the new technology (e.g., how automated alerts prevent missed reassessments).
- Focus:** Emphasize that technology is a safety net for the "**Fifth Vital Sign**" and helps HCWs manage the morbidity risks mentioned in the MMWR.

3. Strategy for Awareness & Strengthening Compliance

- Action:** Launch a "Pain Management Champion" program in each unit to provide real-time peer feedback and troubleshooting.
- Focus:** Use **Visual Management** (e.g., bedside "reassessment clocks" or EMR dashboard indicators) to keep the AIR cycle top-of-mind during busy shifts.

4. Clinical Safety & Patient Advocacy

- Action:** Standardize the use of evidence-based, age-appropriate assessment tools (e.g., Numeric Rating Scale for adults, FLACC for non-verbal patients).
- Focus:** Train staff on identifying "functional goals"—not just a number—such as the patient's ability to deep breathe or move after intervention, which is a key focus of the latest standards.

5. Regulatory Adherence (ADM Gen 01-033 & JCI/CBAHI)

- Action:** Perform a "Gap Analysis" specifically against the **January 2025 JCI 8th Edition** requirements for patient-centered care and healthcare technology.
- Focus:** Ensure every pain intervention has a linked, timed reassessment in the EMR to meet the standards.

Strategic Partnership: DQS-PMU/RMU & Data Management

1. Role of PMU/RMU

The NQI Unit works in direct alignment with the two offices to bridge the gap between clinical bedside practice and digital data. This collaboration ensures that the **349** incidents reported this quarter of the year are not just numbers, but actionable insights.

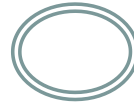
2. Key Contributions

Data Accuracy & Validation: Health Information Department (in collaboration with Nursing Health Informatics) provides the technical infrastructure to validate data (extracted from HIS), ensuring that quarterly reports are credible for decision-making.

Real-Time Dashboards: Developed operational dashboards that allow Nursing Administration to monitor **Nursing Sensitive Indicators (NSIs)** in real-time, facilitating faster intervention for issues like falls or IV complications.

Technology Integration: Supported the rollout of the ivWatch system by ensuring the digital alerts are integrated into nursing workflows, reducing the time spent on manual documentation and increasing time for direct patient care

Conclusion: Our Commitment to a Just Culture



The "No Blame" Philosophy

System over Individual: Shifting the focus from "Who did it?" to "Why did the system fail?"

Psychological Safety: Empowering nurses to report 349 incidents without fear of retaliation.

Shared Responsibility & Accountability

Nurses as Process Owners: Every HIRS report is a clinical trigger for multidisciplinary improvement.

Data-Driven Safety: Leveraging Health Informatics and ivWatch technology to build "error-proof" workflows.

Moving Forward

Continuing to provide the tools, technology, and supportive culture needed to sustain the highest standards of patient-centered care at **KFHU**.

"Quality is never an accident; it is always the result of high intention, sincere effort, intelligent direction, and skillful execution."

THANK YOU